

25 May 2023

Lucy Letby - Cross-examination Day 5: Babies H & I

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MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

Thursday, 25 May 2023

(10.30 am)

(In the presence of the jury)

MS LUCY LETBY (continued)

Cross-examination by MR JOHNSON (continued)

MR JUSTICE GOSS: Mr Johnson.

MR JOHNSON: My Lord.

Overnight have you thought of anything that you have previously said that you would like to amend or add extra detail to?

LETBY: The only thing -- you asked me yesterday if the Datix had been completed about the chest drains and I can confirm it was done.

JOHNSON: Right. And that's by you?

LETBY: Yes.

JOHNSON: And you remember doing that, do you?

LETBY: Sorry?

JOHNSON: You remember doing that?

LETBY: I have seen the Datix on the system.

JOHNSON: Right.

We had got to count 10, which I can see from here is still open on the desk in front of you. I don't know whether the jury have it. It's behind divider 1 in jury bundle 1. This is an allegation of attempted murder on 26 September 2015.

You were, of course, on duty on the night shift of the 25th into the 26th, weren't you?

LETBY: Yes.

JOHNSON: And I'd like to start, if we may, with the population distribution chart, which is in the [Baby H] sequence at tile 177.

(10.32 am)

(Video feed to court disrupted)

(10.36 am)

MR JUSTICE GOSS: I think if you just rewind one question and go back over it just in case -- I don't know whether it did -- I'm not following the transcript.

MR JOHNSON: I have the transcript here and there are actually two issues that I've raised that haven't been transcribed so I'll deal with them both if I may.

MR JUSTICE GOSS: Thank you.

MR JOHNSON: Just so that you understand, Ms Letby, the audio-video link to the transcription service was lost whilst I was asking you questions.

LETBY: Yes.

JOHNSON: So I'm just going to deal with the issues that are missing from the transcript. The first was -- I was asking you whether the staffing levels had in any way, in your view, contributed to [Baby H]'s collapse on this shift?

LETBY: No.

JOHNSON: The second was whether somebody's medical incompetence had contributed in any way to [Baby H]'s collapse.

LETBY: Yes, I think potentially with the management of her chest drains, yes.

JOHNSON: And by management, what do you mean?

LETBY: The insertion of them and how they were secured.

JOHNSON: Right. What is it about the insertion of them that you suggest was deficient?

LETBY: I'm not saying it was deficient, but I think it has been accepted that there were issues with the drains throughout the trial. It's come to light.

JOHNSON: That's not the question. The question, if you wouldn't mind, is what are you suggesting it was about the management, the insertion, to use --

LETBY: That they may not have been correct because there was a lot of discussion needed on how to insert the drains.

JOHNSON: Does it come to this then, because you are saying there was a lot of discussion amongst the doctors about inserting the drains, that means that there is a problem with the drains?

LETBY: Potentially, yes.

MR MYERS: May I just confirm one matter --

MR JUSTICE GOSS: Yes, certainly.

MR MYERS: Because I took a note before we went offline and the note I have was in answer to the question about the competence, the answer we got was:

"There was potential incompetence with regard to the location of the chest drains and how they were secured."

That was what was said before we went offline and certainly that's the note I have, so it's important that although the transcript didn't get that the first time round I note that that is what Ms Letby said in answer to that question at the beginning.

MR JUSTICE GOSS: Yes, thank you.

MR JOHNSON: So that I can understand, we can all understand what you are suggesting, what are you suggesting was the problem?

LETBY: That potentially [Baby H]'s pneumothoraces were not treated appropriately.

JOHNSON: Right. And that is because of what?

LETBY: Lack of experience and using multiple drains of different types that nobody seemed particularly confident to use.

JOHNSON: Which particular doctor or doctors are you suggesting lacked the experience required?

LETBY: Multiple doctors.

JOHNSON: Well, yes, but --

LETBY: Dr Harkness. Dr Jayaram. Dr Gibbs. Dr Ventress even, yes.

JOHNSON: Now, can we go back to tile 27, please? This is something that you dealt with in answer to questions from your own counsel some time ago now. We can see that it's dated a couple of days earlier, on 24 September at 09.15.

LETBY: Yes.

JOHNSON: Just so that we can understand the connection or the relevance or potential relevance of this text sent by you to your friend [Nurse E], are you suggesting there was anything unsafe going on in the unit on the shift that we are dealing with now? In other words, the shift that began on the 25th into 26 September.

LETBY: No, this is referring to a different time.

JOHNSON: So it's not relevant, is that right, to [Baby H]'s collapse?

LETBY: No, because she was one-to-one nursed at that point.

JOHNSON: Yes. Now, a short time ago when I was asking you about your friend [Nurse A], and I was making the point with you that she was truly supernumerary, you suggested that she was in some way responsible for helping you with [Baby H].

LETBY: Yes.

JOHNSON: What are you suggesting about her function on --

LETBY: That the role of a supernumerary shift leader is to support staff on the unit with medications and anything that would need to be done, and particularly with [Baby H] I was unfamiliar with chest drains and [Nurse A] played a part in assisting me with [Baby H]'s care.

JOHNSON: Right. Did you not have experience of chest drains?

LETBY: Not a lot of experience, no.

JOHNSON: I thought you said you'd dealt with them at Liverpool.

LETBY: I have, yes, but I'd not had a lot of experience with them at the Countess of Chester, no.

JOHNSON: Did you suggest to anybody at the beginning of this particular shift that in some way your knowledge or experience was deficient?

LETBY: No. I don't think it was deficient. I think it was raised that I didn't have a lot of experience and part of that was that [Nurse A] was supernumerary and would be able to assist me.

JOHNSON: And what are the tasks with which she was assisting you?

LETBY: Well, I can't recall specifically now, but it would have been medications and I know when she had further chest drains put in, [Nurse A] helped with the insertion and setting-up of those drains with me.

JOHNSON: Right. Let's deal with those two issues then.

Medication she'd have to co-sign with you?

LETBY: Somebody would have had to have co-signed medication with me, yes.

JOHNSON: And that would be true of any child, wouldn't it?

LETBY: Yes.

JOHNSON: So there's nothing exceptional about [Baby H] in that respect?

LETBY: No.

JOHNSON: So far as the insertion of a chest drain was concerned, which particular chest drain are you suggesting she assisted you with?

LETBY: The drain that was inserted on the night of the 26th.

JOHNSON: Yes, this is Dr Gibbs' chest drain; is that right?

LETBY: Yes.

JOHNSON: So this was at 02.15; is that right?

LETBY: Yes, I don't know the time right now, but...

JOHNSON: I think there are two times in the documents: one is 02.10, one is 02.15, in the early hours of the 26th.

LETBY: Okay.

JOHNSON: What I would like you to concentrate on, if you would, please, is -- that was after [Baby H] had collapsed, wasn't it?

LETBY: Yes.

JOHNSON: And what I'm interested in asking you about, and what the jury may be interested in, is what led to her collapse. All right?

LETBY: Okay.

JOHNSON: So let's concentrate on that time if we can, so between 20.00 hours and 02.00 hours or thereabouts.

LETBY: Yes.

JOHNSON: We've got a period of about 6 hours, give or take; yes?

LETBY: Yes.

JOHNSON: Does it come to this, that so far as the assistance you were getting from [Nurse A] was concerned, that was limited to co-signing for medications?

LETBY: I can't say without reviewing the charts.

JOHNSON: Well, have you looked -- you clearly looked at the charts last night because you raised an issue when I asked you to clarify anything you've already said.

LETBY: Yes.

JOHNSON: Have you looked at the charts overnight?

LETBY: Yes, but I can't remember every detail and I think it would be unrealistic if I could remember every single detail.

JOHNSON: Well, you tell -- you see, it's your case that she was in that -- you said in answer to your own counsel, or you gave the impression, that she was in nursery 1 with you, helping you with [Baby H] all the time.

LETBY: I did not say she was there at all times, no.

JOHNSON: What are you saying so we understand?

LETBY: That [Nurse A] was supernumerary and she did help me throughout the evening. I can't be specific at what time or what she actually did at certain times but she was with me on and off throughout the shift.

JOHNSON: And that would all be documented on the charts, wouldn't it?

LETBY: Not all of it, no.

JOHNSON: Well, what sort of thing wouldn't --

LETBY: If she's with me to give verbal advice or if she's helping me do something that I have then signed for on the chart it would not necessarily have her signature.

But she gave me a lot of verbal advice that night.

JOHNSON: About what?

LETBY: About the management of [Baby H].

JOHNSON: In particular in what respect?

LETBY: In management of her chest drains. And then when she collapsed, it was information about baptism and things like that.

JOHNSON: For the time being, let's concentrate on what caused [Baby H] to collapse; all right?

LETBY: Okay.

JOHNSON: Let's not worry about what happened after she collapsed for now. Do you remember that [Baby H] collapsed in the early hours of the 26th?

LETBY: I don't recall specific details. I know she did collapse, yes.

JOHNSON: Do you remember hearing the evidence of [Father of Baby H], who is [Baby H]'s father?

LETBY: Not in detail, no.

JOHNSON: Well, it was read to the jury by agreement on 18 January this year. And of course, this is a statement that you have had available to you for a couple of years now, isn't it?

LETBY: Yes.

JOHNSON: Yes. What [Father of Baby H] said -- and I'm going to read to you directly from his statement; all right?

LETBY: Okay.

JOHNSON: He was talking about his daughter and he said:

"She was in an incubator and was on CPAP to help her breathe. So we spent time in there from Tuesday onwards and on the Friday..."

And we're talking about a Friday, aren't we, 25 September?

LETBY: Yes.

JOHNSON: "... on the Friday, I'd been there late with [Mother of Baby H]'s mum."

[Mother of Baby H]'s mum is [Father of Baby H]'s mother-in-law given [Mother of Baby H] is his wife:

"I'd been there late with [Mother of Baby H]'s mum [on Friday] until about midnight and we'd come back to the house and I was awoken by her in the early hours."

Okay?

LETBY: Okay.

JOHNSON: So that's [Father of Baby H]'s agreed evidence that on this shift, for which we saw the population distribution at tile 177, he was there until just before midnight or thereabouts.

LETBY: Yes.

JOHNSON: Do you remember what time the doctors were called to [Baby H]?

LETBY: No, I don't.

JOHNSON: That night for the first time?

LETBY: No.

JOHNSON: Just before midnight, at 23.50.

LETBY: Okay.

JOHNSON: I would like to look at your nursing note for this occasion. It's tile 208, please. Given that it's your note, would you read it through for us to remind yourself of what it was you wrote?

LETBY: "Written for care given from 20.00 to present. Emergency equipment checked, fluids calculated. [Baby H] nursed in an incubator, ventilated with size 3.5 ET tube, 8 centimetres at lips. Ventilator settings gradually increased during shift in line with blood gas and clinical picture: currently 24/4,

inspiratory time 0.45, rate 60 in 28% oxygen. x2 chest drains in situ at start of shift. Intermittently swinging. Serous fluid ++ accumulating.

"23.30. Bradycardia and desaturation requiring Neopuff in 100% to recover. 10ml [I assume that's air] aspirated from chest drain by Registrar Ventress. Following poor blood gas and 100% oxygen requirement, Consultant Gibbs attended the unit and inserted a third chest drain."

JOHNSON: Let's just stop it there because that takes us up to and including the time of the first desaturation and indeed the second desaturation when Dr Gibbs was called to help. All right?

LETBY: Okay.

JOHNSON: Do you agree with that?

LETBY: Yes.

JOHNSON: Can we look at the time that you created that record, please. Can you confirm that you began to write it at 04.14 and you closed it at 04.28?

LETBY: Yes.

JOHNSON: And I am going to suggest to you that you misrepresented the time of this event; okay?

LETBY: Okay.

JOHNSON: So I'm going to suggest this is another occasion where you have deliberately misrepresented the facts; okay?

LETBY: Okay.

JOHNSON: This is an event that happened just before midnight, at about 23.50, isn't it?

LETBY: I don't recall. From my notes there I've said 23.30.

JOHNSON: Where would you have got that information from when you wrote it down at 04.14?

LETBY: Either notes that I've made myself or any medical records --

JOHNSON: Yes.

LETBY: -- or anything on the charts relevant to that time.

JOHNSON: We've seen many medical records, haven't we --

LETBY: Yes.

JOHNSON: -- written in varying degrees of legibility by varying medical practitioners?

LETBY: Yes.

JOHNSON: And they were available on paper for you to consult if there was any doubt in your mind about when something had happened?

LETBY: Yes.

JOHNSON: Dr Ventress, for it was she who was called at 23.50, had made a note and you could have looked at it and used it to create your own note, couldn't you?

LETBY: At the time, if she had written the note prior to this, then yes.

JOHNSON: All right. Let's see what you were writing on the manuscript notes because these will be relevant as well.

If we can go to tile 199, please. I'd like to concentrate on the material in the left-hand side of the page if we can. This actually should be material that the jury has in jury bundle 2. If you'd like to look at a paper copy, you will find it behind divider 10.

The page number, as we can see on the screen, is 12701. Would you prefer a paper copy or are you happy with the screen?

LETBY: No, I'd prefer to see it on the screen, thank you.

JOHNSON: Okay. The utility of the paper copy in these circumstances may be to tie in the times with the entries, okay? Do you understand what I mean?

LETBY: Yes.

JOHNSON: Because in order to read what's on the screen it may be, for people like me, we're going to have to magnify what you have written and that will obliterate the corresponding time at the top of the screen. All right?

LETBY: Okay.

JOHNSON: I'd like to start, with Mr Murphy's assistance, please, with your entries, which are the four in the left-hand half of the page to the right-hand edge of that half.

If we can turn them through 90 degrees as well, Mr Murphy, clockwise, then it may facilitate us being able to read them.

Do we see the first entry that you made as being at 21.00 hours?

LETBY: Yes.

JOHNSON: The second in the 22.00 hours column?

LETBY: Yes.

JOHNSON: What does that read, please?

LETBY: "22.10. Desat 52% during..."

JOHNSON: "Heel prick"?

LETBY: Yeah:

"... heel prick. SHO present. Serous fluid ++. x2 drains."

JOHNSON: Yes, "SHO present". Who was the SHO that you were referring to here?

LETBY: I can't recall from memory.

JOHNSON: Well, there was an SHO on duty that night called Jessica Burke.

LETBY: Okay.

JOHNSON: We saw it in the -- we can see it at tile 177.

LETBY: Okay.

JOHNSON: Sorry, not Jessica Burke, Jessica Scott, I beg your pardon, Jessica Scott.

The SHO wasn't present, was she?

LETBY: Yes, that's what I've written there.

JOHNSON: Is there a note in the medical notes from the SHO about this desaturation?

LETBY: I don't know.

JOHNSON: Oh, I think you do. There isn't, is there?

LETBY: I don't know.

JOHNSON: "Serous fluid ++, x2 drain."

Is that what --

LETBY: Yes.

JOHNSON: You then write in the 23.00 hours column:

"Blood commenced 23.05."

LETBY: Yes.

JOHNSON: Is that your writing as well?

LETBY: Yes.

JOHNSON: Then in the midnight column:

"Brady/desat 23.30."

LETBY: Yes.

JOHNSON: "10ml air aspirated from..."

LETBY: "Flutter drain."

JOHNSON: "... from flutter drain."

And then there's an upward arrow?

LETBY: Yes, it's increased the ventilator pressure to 24/4.

JOHNSON: Then to the right of that what does it say in the next box?

LETBY: "Clear."

JOHNSON: Well --

LETBY: All of the writing?

JOHNSON: You interpret whether you do them individually as different boxes or as a unit, the totality of that material.

LETBY: I can't see without looking at the columns. I'm assuming it's clear either orally or -- I can't see the column it

represents. But there's small blood -- "small bloodstained" and then beneath that it says "clear".

JOHNSON: Yes. Is it "plus clear"?

LETBY: "Plus small bloodstained plus clear."

But I'm not sure which column that refers to.

JOHNSON: It's all in the -- we're looking at things sideways on now, whereas this is a vertical column. This is why it helps to look at the paper copy at the same time so that we can orientate what we're looking at. This is all in the midnight column.

LETBY: Yes. I'm talking about the row that would be on the left-hand side, I think, where it indicates if it's OP -- here we are, "suction". The top bit is:

"Suction on ET produced plus small bloodstained. Oropharyngeal was plus clear."

JOHNSON: So that's looking into the mouth is clear?

LETBY: The airway was clear, I've had clear fluid.

JOHNSON: So here we have another child under your care who's producing bloodstained secretions in their throat; is that right?

LETBY: I think if it's come from the ET tube, it wouldn't necessarily be the throat. If the ET tube is in the lungs, I presume it's from there.

JOHNSON: Yes. Had you interfered with [Baby H]'s ET tube?

LETBY: No.

JOHNSON: Moved it around a bit to destabilise her?

LETBY: No.

JOHNSON: Can we look at your defence statement, please, to try and put this into a bit more context. Have you got the document?

LETBY: Yes, it's here.

JOHNSON: This is paragraph 101, isn't it?

LETBY: Yes.

JOHNSON: What did you write of these events on this night?

LETBY: "I don't recall any specific episode of desaturation during the night shift of the 25th into the 26 September."

JOHNSON: Yes. You then go on, I think, as we established yesterday, to say that the following night, the 26th to the 27th, [Baby H] deteriorated rapidly and you were with [Nurse A].

LETBY: Yes, that date is not correct.

JOHNSON: No, okay. Is that actually a reference to this night, the 25th to the 26th?

LETBY: Yes.

JOHNSON: And to be fair to you, you then say at 103:

"In my memory, both nights merge into one. I cannot be specific about what I did during either of the collapses."

Then you talk about the baby LO that we dealt with yesterday and to whom we will come in due course.

LETBY: Yes.

JOHNSON: Of course, just going back to your note at 22.10, if a baby with two chest drains in, as [Baby H] was, had really desaturated to 52%, that would be potentially a serious medical issue, wouldn't it?

LETBY: Yes, if she didn't correct, yes.

JOHNSON: But you would call a doctor, wouldn't you?

LETBY: An SHO is present.

JOHNSON: Well, I know that's what you say. But I'm suggesting to you because there's no note, an SHO wasn't present and you have cooked the records.

LETBY: I don't agree.

JOHNSON: And you are creating, I suggest, in the handwritten documentation the impression that [Baby H] was deteriorating in the lead-up to her more serious crash.

LETBY: No.

JOHNSON: Just going back to your nursing note at tile 208, please. We've read through this once, but if we can expand the top -- thank you. And down a bit.

Where is mention of this earlier desaturation before 23.30 --

LETBY: There isn't.

JOHNSON: -- with the SHO present?

LETBY: There isn't.

JOHNSON: Why not?

LETBY: Not every single thing gets written down. It's not been written down and that's an error on my part.

JOHNSON: A serious error, isn't it, if it happened?

LETBY: But the SHO was present.

JOHNSON: Well, I know that's what you've written on a manuscript document but there's no --

LETBY: Any member of staff --

JOHNSON: Hold on, hold on. There's no telling when you actually wrote that detail into that document, is there?

LETBY: On to the handwritten document?

JOHNSON: Mm.

LETBY: No.

JOHNSON: There is telling when you wrote detail into this document though, isn't there --

LETBY: Yes.

JOHNSON: -- because we have an opening and a closing time?

LETBY: Yes.

JOHNSON: You wrote that detail on the handwritten document after [Baby H] had seriously desaturated and had a cardiac arrest, didn't you?

LETBY: No.

JOHNSON: And that is the explanation for why it doesn't appear in your nursing notes?

LETBY: No. Those charts are available to everybody to see. If it was blank or it had been added to later, that would have been noticed.

JOHNSON: Noticed by who?

LETBY: Other staff.

JOHNSON: When?

LETBY: The doctors would be looking at the charts, other staff nurses look at the charts.

JOHNSON: You're making this up as you're going along, aren't you?

LETBY: No.

JOHNSON: You wrote that in after [Baby H] had collapsed?

LETBY: No.

JOHNSON: That's why you haven't written the name of the SHO, as with Dr Wood, in the case of [Baby E] --

LETBY: No.

JOHNSON: -- isn't it?

LETBY: No, it's common practice just to write SHO or reg.

JOHNSON: Your common practice?

LETBY: It can be, yes. I don't think every baby I've ever looked after every time I've written the name of doctor.

JOHNSON: Often you don't even write SHO or reg, do you, you write doctors?

LETBY: At times, yes, which all staff would do.

JOHNSON: But in your case it seems to happen when babies collapse, doesn't it?

LETBY: I can't answer that. We're only looking at collapsed babies, we're not looking at my whole work over the duration of 4 years.

JOHNSON: Do you remember [Father of Baby H]'s evidence that was agreed?

LETBY: Not in clarity right now, no.

JOHNSON: I just reminded you of it.

LETBY: Oh yes, that bit, yes.

JOHNSON: He left just before midnight.

LETBY: Yes.

JOHNSON: No mention of seeing his baby collapsing.

LETBY: No.

JOHNSON: No mention of a doctor being there. This is your lies in a document, isn't it?

LETBY: It's not lies, no.

JOHNSON: Let's go to Dr Ventress' notes, please, at tile 210.

We can see there the time that Dr Ventress has put on this, can't we?

LETBY: Yes.

JOHNSON: The same time as [Father of Baby H] in his agreed evidence said he left.

LETBY: Yes.

JOHNSON: About the same time. And what you were telling her is:

"Several episodes of desaturation in past 2 hours."

Isn't it?

LETBY: Yes.

JOHNSON: The first one after gas is taken?

LETBY: Yes, so she desaturated during the heel prick, yes.

JOHNSON: "Became agitated" is what you told Dr Ventress, though, isn't it?

LETBY: I can't be specific on what wording I used.

JOHNSON: Well, what has she written?

LETBY: She has written "became agitated".

JOHNSON: Is that desaturating down to 52%, becoming agitated?

LETBY: I don't know what you mean.

JOHNSON: Well, are they consistent with each other, those two descriptions? One, she became agitated. The other, desaturating to 52%.

LETBY: Yes, because babies like [Baby H] don't handle very well.

JOHNSON: They don't become moribund, lifeless, when they desaturate?

LETBY: No.

JOHNSON: No?

LETBY: Not every time, no.

JOHNSON: Not every time, just sometimes?

LETBY: Yes.

JOHNSON: "One episode associated with poor chest movement and bradycardia. Suctioned ET and thick mucus suctioned."

Which of the episodes that we've just looked at on the chart that we were looking at is that?

LETBY: The part where I read out the secretions. I'm not sure what time that was.

JOHNSON: That's in the midnight column, so --

LETBY: Okay.

JOHNSON: That's this time.

LETBY: Right.

JOHNSON: It doesn't seem to be therefore what you're telling Dr Ventress, does it, if she's recorded what you told her correctly?

LETBY: I don't know that this is what I've told her though.

Was she possibly there for this? I don't know.

JOHNSON: Well, we heard from her, she gave evidence, she talked us through this note. This is clearly what you are telling her,

isn't it? You are reporting what you are saying has happened; do you agree?

LETBY: No, I couldn't say that specifically, no.

JOHNSON: Who else would have been telling Dr Ventress what had been happening with [Baby H] in the preceding 6 hours?

LETBY: If the SHO was present, which I believe they were, and also [Nurse A] was with me at times.

JOHNSON: So you're saying the SHO is here at this time?

LETBY: No, I'm saying I don't know.

JOHNSON: "Further episodes. No change in heart rate. Recovered with bagging on higher pressures."

Where are they in your handwritten notes?

LETBY: I believe I did write that the PIP was increased to 24/4 on the handwritten column.

JOHNSON: Yes, and that's at midnight.

LETBY: Is it? Okay.

JOHNSON: Well, you know it is because we've just looked at it.

LETBY: Okay.

JOHNSON: Are you trying it on?

LETBY: No.

JOHNSON: Then is that:

"Fractional oxygen reduced to 30% between episodes"?

LETBY: Yes.

JOHNSON: What's that a reference to, what were you telling Dr Ventress?

LETBY: I can't confirm that I was telling her anything but that reads that her oxygen requirement had gone down to 30% in between episodes.

JOHNSON: Given that [Father of Baby H] had only just left at this time, according to the agreed evidence, I am suggesting that you were falsely creating the impression to the registrar,

your friend, Dr Ventress, that [Baby H] was a child who had been presenting problems over the preceding hours. What do you say to that?

LETBY: Can you say that again?

JOHNSON: Yes. I'll break it down. You heard me remind you of the agreed evidence from [Father of Baby H] that he left at just before midnight?

LETBY: Yes.

JOHNSON: This account was being given to Registrar Ventress at just before midnight, wasn't it?

LETBY: Yes.

JOHNSON: And I am suggesting that you were falsely creating the impression to Dr Ventress that [Baby H] was a child who had been having problems for a couple of hours by this stage. What do you say?

LETBY: No, I don't agree that it was false, no.

JOHNSON: It's just that your nursing note that's typed, and your handwritten notes on the chart, don't match up and that's an oversight?

LETBY: Yes.

JOHNSON: And it's an innocent coincidence that this all happened just after, apparently, [Father of Baby H] had just left?

LETBY: Yes.

JOHNSON: Yes. If we look at the observations chart, please, and we've got this in paper format, it's page 12696. If Mr Murphy would help by -- tile 178 should have this material on it or 195.

MR JUSTICE GOSS: 195 and 196 are the first two pages in this section, section 11.

MR JOHNSON: Yes.

MR JUSTICE GOSS: Section 10, sorry.

MR JOHNSON: Although it covers counts 10 and 11.

MR JUSTICE GOSS: Exactly. There isn't a 11.

LETBY: I think my section 10 here is [Babies O & P].

MR JOHNSON: If you'd ask the officer just to hand it to me, just that file, I'll see what's gone wrong. We've got the wrong file. It's file 2, please.

(Pause)

If you could turn on a page, please, to 12696. Do we see any issues recorded in this chart between the beginning of your shift at 20.00 hours and midnight?

LETBY: No.

JOHNSON: Any reason for that?

LETBY: What do you mean?

JOHNSON: Well, according to the information you were giving Registrar Ventress, this was a child that had been having significant issues of significant desaturations.

Should there be anything on this chart?

LETBY: No. This reflects that specific moment in time.

JOHNSON: So at the moment in time that you were taking observations, everything was fine with [Baby H]?

LETBY: The observations that I've taken here, yes.

JOHNSON: Okay. To avoid confusion, if we put that away for a second, please, and if we can go to tile 210.

If we go down a little bit more, thank you.

Right at the end of the note, we have something that ties in to a point that you raised at the beginning of evidence this morning. It's the very last note that Dr Ventress made and it says:

"Second chest drain..."

And in this context, just to remind us all, this is Dr Jayaram's chest drain at the seventh intercostal space, isn't it?

LETBY: Yes.

JOHNSON: "... advanced back in to 4 centimetres as was almost out."

LETBY: Yes.

JOHNSON: "Done prior to chest X-ray."

And so just after the time -- it follows, doesn't it, that just after the time that [Father of Baby H] had left his daughter, it was noticed by Dr Ventress that the second chest drain had almost come out?

LETBY: Yes.

JOHNSON: Why hadn't you noticed that?

LETBY: It's not a nursing role to be able to measure the exact distance of a drain. Medical staff put drains in.

JOHNSON: They do, but you have already told us that in your view one of the potential problems with the chest drains in [Baby H]'s case arose because they weren't stitched in, they were taped in.

LETBY: Yes.

JOHNSON: And you knew that at Liverpool, from your training, they stitch them in.

LETBY: Yes.

JOHNSON: And just remind us what you told us yesterday was the risk that you perceived as being associated with taping rather than stitching?

LETBY: The drains are less secure.

JOHNSON: Exactly. And all this you knew?

LETBY: Yes.

JOHNSON: And given that this was a child with chest drains, that leads to my next question: why were you not checking the drains?

LETBY: I was checking the drains.

JOHNSON: The answer, I'm going to suggest, is because you removed the drain.

LETBY: No.

JOHNSON: And that is the reason why [Baby H] desaturated just before midnight, just after her father had left.

LETBY: No.

JOHNSON: Because you were sabotaging [Baby H] that night, weren't you?

LETBY: No.

JOHNSON: Let's look at the blood transfusion. Can we go to tile 261, please. This was something that your counsel, Mr Myers, asked you about on 15 May.

LETBY: Yes.

JOHNSON: Do you remember?

LETBY: Yes.

JOHNSON: Do you remember what you said to him about your nursing entry?

LETBY: That the time was incorrect.

JOHNSON: In what respect is the time incorrect?

LETBY: That the transfusion was completed at 02.00.

JOHNSON: Whereas the truth is?

LETBY: 03.00.

JOHNSON: So if we're looking to find the mistake, it's about five or six lines up from the bottom of the page as we see it on the screen.

LETBY: Yes.

JOHNSON: Is that right?

LETBY: Yes.

JOHNSON: That was something that you wrote between 04.14 and 04.28?

LETBY: Yes.

JOHNSON: Where did you get the information from?

LETBY: For the timings?

JOHNSON: Yes.

LETBY: It would have been from the charts and I've miswritten it down, yes.

JOHNSON: You what, sorry?

LETBY: I've miswritten the time down.

JOHNSON: Where have you miswritten the time?

LETBY: Here in the nursing notes.

JOHNSON: Anywhere else?

LETBY: No.

JOHNSON: Can we look at tile 239, please? Look in the top right-hand box. What's written there?

LETBY: It looks like it started at 02.05 and it's been corrected to 03.05.

JOHNSON: Yes. And whose writing is that?

LETBY: It's mine.

JOHNSON: Yes. So the same mistake but in two different places?

LETBY: Yes.

JOHNSON: When did you correct the handwritten version?

LETBY: I couldn't say, I couldn't answer that. [Nurse A] and I would both have needed to sign the reverse of that chart when it was finished, so presumably we both were there when that was written.

JOHNSON: When you say the reverse, do you mean the other side?

LETBY: Yes.

JOHNSON: So the back of it?

LETBY: Yes. There's columns to say that you've documented the time when it's finished, yes.

JOHNSON: So [Nurse A] signing that wouldn't see what you had written on the front of it?

LETBY: Yes. That's what she's signing to say, that she's seen the time we've stopped the infusion.

JOHNSON: Well, I know, but sometimes people when presented with a document from somebody they trust will sign it on trust, won't they?

LETBY: You'd have to talk to them about that. That's on them, not me.

JOHNSON: Okay. On them, not you?

LETBY: If you're saying that staff would check a form without looking at it then that's -- I can't answer for somebody else.

JOHNSON: Have you ever signed a form on trust from one of your colleagues?

LETBY: No, I don't believe I have, no.

JOHNSON: No. So can we take it that you changed that some time after you entered the nursing notes into the Meditech system?

LETBY: I couldn't say with clarity.

JOHNSON: Well, it's the obvious explanation, isn't it?

LETBY: Yes.

JOHNSON: Yes. What happened just after 03.05?

LETBY: I don't recall.

JOHNSON: Really?

LETBY: No, I don't recall the exact timings of everything that's happened, no.

JOHNSON: Your child for whom you were responsible had a cardiac arrest --

LETBY: Okay.

JOHNSON: -- [Baby H]. And that document there that we see on the screen shows that you were very closely concerned with [Baby H] just before she had that cardiac arrest, doesn't it?

LETBY: Yes.

JOHNSON: Or at least it does in its corrected state, but perhaps not in its original state. Do you agree with that?

LETBY: No, because I think the blood transfusion observations and the drip readings do confirm that it was at 03.00.

JOHNSON: If that's the case how on earth did you ever write in 02.05?

LETBY: Because we're human and people make mistakes.

JOHNSON: Would you agree this is yet another apparent anomaly in the paperwork at the time just at which a child has a cardiac arrest?

LETBY: I agree that the time is wrong, yes. But on the observation charts and the drip reading charts, the timings were correct.

JOHNSON: And whose responsibility is the error?

LETBY: Mine, in my nursing notes, mine.

JOHNSON: [Baby H] had to have CPR; do you remember that?

LETBY: Yes.

JOHNSON: Do you remember she had to have adrenaline to bring her back?

LETBY: Yes.

JOHNSON: Do you agree this was a very, very serious event?

LETBY: Yes.

JOHNSON: Do you remember [Baby H]'s dad being there when this happened?

LETBY: I can't say from memory, no.

JOHNSON: Well, I'm going to remind you what he said about this and ask whether you remember this:

"I remember the mottling was running out of her skin towards her fingers."

That was in the agreed evidence of [Father of Baby H]. Do you agree with that?

LETBY: Yes, I agree she was mottled, yes.

JOHNSON: Do you agree that it was moving?

LETBY: No, I don't agree it was moving, no. It was all over her body though.

JOHNSON: "Running out of her skin towards her fingers."

LETBY: I don't agree with that.

JOHNSON: This event was so serious that you thought [Baby H] was brain damaged, didn't you?

LETBY: No, there was a discussion with the parents about whether [Baby H] could have sustained brain damage.

JOHNSON: Yes. That was the concern, so serious was this?

LETBY: Yes.

JOHNSON: And yet, if we go to tile 277, please --

MR JUSTICE GOSS: Mr Johnson, sorry for my benefit, could you remind me behind what tile this chart is?

MR JOHNSON: The one that's on the screen?

MR JUSTICE GOSS: J12651.

MR JOHNSON: I can: it's 239.

MR JUSTICE GOSS: Thank you.

MR JOHNSON: So these are [Baby H]'s blood gases, which we also have in paper format, also behind divider 10, and these ones are page 12709. If we scroll down to get the 26th, please. Stop there. Thank you.

We see that the three blood gases on the night shift at the beginning of the 26th are all in your writing, aren't they?

LETBY: Yes.

JOHNSON: Each signed by you?

LETBY: Yes.

JOHNSON: And do we see that by 6.44, [Baby H] has a pretty good blood gas?

LETBY: Yes, it's improved, yes.

JOHNSON: Well, it's a good blood gas, isn't it?

LETBY: Yes.

JOHNSON: So from having been at the brink of fatal collapse about 3 or 4 hours earlier, she had got back to a position -- she'd had a miraculous recovery, hadn't she?

LETBY: Yes.

JOHNSON: Were you pleased?

LETBY: Of course I was pleased.

JOHNSON: Or were you frustrated by the fact that your attempt to kill her had failed?

LETBY: No.

JOHNSON: The day shift that followed was 26 September and you handed over [Baby H] to Kathryn Percival-Calderbank, didn't you?

LETBY: I believe so, yes.

JOHNSON: If we can just go to, just to remind you, put you back in the moment, if we can, tile 400. This is a note that you sent to your friend [Nurse A], who'd been on with you that night; is that right? The previous night shift, I should say.

LETBY: Yes.

JOHNSON: Can you just read it out for us, please?

LETBY: "[Baby H] had good day. 1 drain removed but just blocked tube and tonnes of secretions. Shelley has her. [Nurse B] in 4 and seemed a bit off when she came in, saying she'd have liked to have had her but now keeps saying she doesn't feel well. I have E and N. I hope you are okay."

JOHNSON: Then the next tile, please. Again, one of yours, I believe. This time to your friend Dr Ventress.

LETBY: Yes.

JOHNSON: Can you help us with it, please?

LETBY: "I think so too."

I'm not sure what that is in relation to:

"She's had a stable day. Took original drain out at 8, just blocked tube. Loads of secretions! I have E and N. [Nurse B] in 4."

JOHNSON: Right. And these are texts that you are sending whilst on duty on the following night shift, aren't they?

LETBY: Yes.

JOHNSON: This is the shift on which the other child, the very seriously ill child, came in towards the end of the night shift; is that right?

LETBY: Yes.

JOHNSON: Okay. Let's do part of this shift before the break if we can. This other child was the child LO; is that right?

LETBY: Yes.

JOHNSON: If we start, please, Mr Murphy, with tile 385. This gives us the population distribution, doesn't it?

LETBY: At the start of the shift, yes.

JOHNSON: Yes. [Baby H] perhaps unsurprisingly still in nursery 1, but still alone, but this time with Shelley Tomlins?

LETBY: Yes.

JOHNSON: You were in nursery 2 with two children, NC and EM?

LETBY: Yes.

JOHNSON: The other nurse in there was with [Baby G], Christopher Booth; is that right?

LETBY: Yes.

JOHNSON: [Nurse B] with four children in nursery 4?

LETBY: Yes.

JOHNSON: And the nursery nurse Elizabeth Marshall with four children, who included [Baby I] --

LETBY: Yes.

JOHNSON: -- in nursery 3? And I think if we just scroll up, the shift leader on this occasion was Christopher Booth.

LETBY: Yes.

JOHNSON: It may, just to bring it back to your mind, be convenient to go to the neonatal review, please, which is in the white large file. The file that looks like this (indicating).

There are two separate reviews for each of these two counts, so it's the second [Baby H] review, please. It's headed "Event 10":

"NNU review schedule. Event 10. [Baby H]."

This is an eleven-page document. If we go to page 2, we see the same population distribution chart that we saw in the electronic sequence; is that right?

LETBY: Yes.

JOHNSON: But if we go to page 11, the final page, we see how things had changed by the end of the shift.

LETBY: That's right, yes.

JOHNSON: The two most relevant changes are, first, the discharge of [Baby H], who had been transported out of Chester --

LETBY: Yes.

JOHNSON: -- by the transport team, and the arrival of LO, who came into the NNU at 05.10?

LETBY: Yes.

JOHNSON: Do you remember going through this document with your counsel, Mr Myers, and suggesting to the jury that you didn't really have much to do with [Baby H] on this particular shift?

LETBY: So my memory at the time is I could not remember when baby LO had been born. Obviously, now we've had the notes I know that he was born at a different time.

I didn't know at the start what time in the shift he had been born.

JOHNSON: Right. Well, I think we may be confusing -- may be at cross-purposes here. Maybe not, maybe we are.

I'm not talking about your defence statement; okay?

LETBY: Okay.

JOHNSON: I'm not talking about your interviews. What I'm talking about is the evidence you gave to the jury on 15 May.

LETBY: Okay.

JOHNSON: I'm going to summarise it in this way for your comment, that you went to some of the entries in this schedule and the gist of what you were saying was that you didn't have much to do with [Baby H] on this shift.

LETBY: Yes.

JOHNSON: Do you accept that as a reasonable summary of what you said?

LETBY: Yes.

JOHNSON: Right. Do you remember also saying that if something wasn't in the notes on this shift then, in effect, you couldn't really remember it?

LETBY: Yes, I was reliant on my notes for a lot of it, yes.

JOHNSON: Yes, and because you're not the designated nurse you didn't make a note and because you didn't make a note, you couldn't remember; is that a fair summary?

LETBY: Yes, not with any great detail, yes, that's right.

JOHNSON: All right. Had you forgotten the evidence of Dr Neame?

LETBY: I can't recall Dr Neame's evidence now, no.

JOHNSON: Well, if we just scroll up, please, Mr Murphy, for the benefit of those watching remotely. It's just on tile 385, please. It's the sequence of events, please.

Dr Neame was the registrar, wasn't he?

LETBY: Yes.

JOHNSON: And Dr Jessica Scott, again the SHO?

LETBY: Yes.

JOHNSON: So on this shift, Dr Neame was in substitution for Dr Ventress, your friend?

LETBY: Yes.

JOHNSON: This is the night, isn't it, where you have got confused in your defence statement between what happened on this night and what had happened the previous night?

LETBY: Yes.

JOHNSON: Let's start with the standard questions I tend to ask you first. Are you suggesting that staffing levels caused or contributed to [Baby H]'s cardiac arrest on this shift?

LETBY: No.

JOHNSON: Are you suggesting that medical competence or incompetence made a contribution?

LETBY: I can't comment because I wasn't the nurse looking after [Baby H] that night.

JOHNSON: Are you suggesting that anybody, whether they be a doctor or a nurse, made a mistake which contributed?

LETBY: No.

JOHNSON: I'm going to suggest to you that this was yet another shift on which you wanted to be in nursery 1.

LETBY: No.

JOHNSON: Let's start by looking at the neonatal review, if we can, please. For the avoidance of doubt, as I said before, we're in neonatal review schedule, event 10.

We see, as we always do with these documents, that any entry for which you have signed appears in blue; do you agree?

LETBY: Yes.

JOHNSON: And any entry relating to the subject of the allegation appears in a yellow-shaded row.

LETBY: Yes.

JOHNSON: So concentrating on [Baby H], please, we see several entries at 20.00 hours made by a combination of Shelley Tomlins and Kathryn Percival-Ward.

LETBY: Yes.

JOHNSON: At 18 and 19, 5 and 15 minutes later respectively, the same nurses making entries for [Baby H].

LETBY: Yes.

JOHNSON: Then at 20 past, line 23, Dr Soni?

LETBY: Yes.

JOHNSON: At 27, Shelley Tomlins' nursing note?

LETBY: Sorry, can I just clarify? It says Kathryn Percival-Ward. Was she on the day shift? She's not on the map.

JOHNSON: No, well, it would follow then, wouldn't it, that she must have been on the day shift?

LETBY: That she stayed on, okay.

JOHNSON: Okay?

LETBY: Okay.

JOHNSON: Do you have an eye for detail?

LETBY: Yes.

JOHNSON: Line 27 at 20.30 hours, Shelley Tomlins making another note --

LETBY: Yes.

JOHNSON: -- regarding a desaturation. Then at 41 to 43, further Shelley Tomlins notes; is that right?

LETBY: Yes.

JOHNSON: 48, 59 and then 61 to 64, all, with the exception of Dr Neame at 21.15, notes made by Shelley Tomlins?

LETBY: Yes.

JOHNSON: But you were involved in the incident at 21.15, weren't you?

LETBY: I can't recall.

JOHNSON: Do you remember what Dr Neame said about it?

LETBY: No, I don't, no.

JOHNSON: Can we go to tile 399, please:

"Fast bleeped..."

So time, 21.15, do you agree?

LETBY: Yes.

JOHNSON: Dr Neame's details?

LETBY: Yes.

JOHNSON: "Fast bleeped to NNU due to desaturation and bradycardia. ETT removed by nursing staff and being bagged."

Do you see that?

LETBY: Yes.

JOHNSON: And reference to the amount of oxygen, 100% oxygen, on a Neopuff.

LETBY: Yes.

JOHNSON: The ETT was blocked with thick yellow/brown secretions?

LETBY: Yes.

JOHNSON: Do you not remember that?

LETBY: No.

JOHNSON: Well, Dr Neame's evidence was that when he arrived, you were the nurse doing this and there was nobody else there.

LETBY: Well, I don't have any recollection of that.

JOHNSON: Let's look at tile 406. You read it for us, please.

LETBY: "I've been helping Shelley so least still involved but haven't got the responsibility. Aw, did you get his boots?"

JOHNSON: The boots is a reference to one of [Nurse A]'s kids who was after some football boots?

LETBY: That's right.

JOHNSON: Nothing to do with this. I'm more interested in the first sentence. What were you talking about?

LETBY: That I'd been helping Shelley at that point.

JOHNSON: With who?

LETBY: I'm assuming it must be with [Baby H].

JOHNSON: Mm. What do you mean by "so least still involved"?

Does that mean "so at least still involved"?

LETBY: Yes, so I'd had [Baby H] for the previous three nights, so I knew [Baby H] well, so it meant I was still involved in her care but I didn't have responsibility.

JOHNSON: Anything that went wrong wasn't going to come back on you, was it?

LETBY: No.

JOHNSON: No.

LETBY: No.

JOHNSON: Do you agree this confirms what Dr Neame told us, you were the person with [Baby H]?

LETBY: No. And I do not believe that I would have removed an ET tube by myself.

JOHNSON: Well, let's go back to the neonatal review. This message is timed at 21.51.

LETBY: Yes.

JOHNSON: Look at page 4. What had you been involved with with [Baby H]? What can we see in that yellow-shaded -- those yellow-shaded lines which relate to [Baby H] together with your blue --

LETBY: None.

MR JOHNSON: Nothing.

My Lord, that may be a good moment for the break.

MR JUSTICE GOSS: Yes, certainly.

15 minutes, please, members of the jury.

(11.44 am)

(A short break)

(11.59 am)

(In the absence of the jury)

MR MYERS: One observation. I was just checking the evidence we've had on what happened when Dr Neame attended the nursery. We understand the requirement to deal with some degree of

summary with the evidence when we're going through it because there are practicalities to keep in mind. But a natural concern of ours is if that summary doesn't identify other matters that can be relevant, and I'll give an illustration.

When Matthew Neame was cross-examined about this issue in this nursery on this night, he agreed that it is possible that it was Shelley Tomlins who told him what had happened, rather than Lucy Letby. And we note in Shelley Tomlins' nursing note she has an entry for 20.30 and a desaturation and we're in the process of just checking what Shelley Tomlins said about it.

But our concern, particularly in a case where this examination is necessarily as lengthy as this one, is that an impression can be created, not intentionally, but can be created by just going to one part of the evidence that something is a given, in this case that Ms Letby is the person who, without debate, is present at the time this takes place.

That isn't the way this came across in the evidence.

It could leave the jury with the wrong impression and also, importantly, leave the defendant with the wrong impression if there are matters that she really cannot remember and then relies upon a summary of the evidence.

Some of this we can deal with in due course, but it is a concern when it is such a long process that if there isn't certain degree of accuracy with some of these matters then the wrong impression can be created.

MR JUSTICE GOSS: Have you discussed this with Mr Johnson, about this particular point?

MR MYERS: Yes, just as we came in because I was just checking it.

MR JUSTICE GOSS: Have you invited Mr Johnson to correct this?

MR MYERS: I haven't suggested any remedial action, my Lord, because I've only just had the reference myself.

MR JUSTICE GOSS: Do you want to just leave it for the moment and come back to it perhaps after the short adjournment and so this can be checked fully?

MR JOHNSON: I've got the reference here. It's 23 January 2023.

MR MYERS: Thank you.

MR JOHNSON: Page 51 in chief:

"Question: Who was doing what when you arrived?

"Answer: My recollection is that Lucy Letby was doing the -- providing the breathing support when I got there.

"Question: Did you speak to her?

"Answer: I would have done. I can't recall much of the conversation. I recalled talking to Lucy Letby on this occasion. That's my recollection."

Then:

"Question: In those circumstances who would ordinarily be the nurse who brought you up to speed with what was happening?

"Answer: Ordinarily it's the nurse looking after the baby who's able to describe what they've observed."

And so on. So that's the first collapse.

The second collapse at 01.30, page 64:

"Question: Do you remember which nurses were present when you arrived?

"Answer: I think again it was Lucy Letby.

"Question: Do you remember anyone else being present at that point?

"Answer: No, I can't say for certain who else was there.

"Question: Do you remember who you spoke to on that occasion when first you arrived?

"Answer: Again I think I spoke to Lucy Letby. I remember a sense of being more concerned on this occasion because it wasn't completely clear why she had deteriorated, there wasn't as clear an explanation. "

So that was the evidence in chief.

In cross-examination at page 80 --

MR JUSTICE GOSS: Sorry, can you just remind me which day this was?

MR JOHNSON: Day 57, 23 January 2023.

Page 80 is cross-examination. A question from Mr Myers:

"Question: One name that you don't mention in either of your statements is Shelley Tomlins; do you know who she was?

"Answer: Yes.

"Question: Do you recall her at all on these occasions?

"Answer: I can't specifically recall that she was helping on these occasions.

"Question: Is that because you have no recollection of her at all?

"Answer: I remember her working on the unit and being involved in care but I don't remember specifically what her role was on these occasions.

"Question: And is it right that you don't have any recollection of her involvement in these occasions?

"Answer: That is correct, yes.

"Question: Do you recall at any point whether, just to be quite -- particularly this, she, Shelley Tomlins -- or did or could have given you a history about what had happened with [Baby H] on either of these occasions?

"Answer: That's possible."

MR JUSTICE GOSS: Yes.

MR JOHNSON: So --

MR MYERS: It goes on:

"Answer: That's possible, yes.

"Question: I want to be quite clear so you can answer this. You said you recall maybe on the first occasion Ms Letby telling you something about what had happened. You've given that evidence, haven't you?

"Answer: Yes.

"Question: Is it possible that in fact it's Shelley Tomlins who told you what had happened rather than Lucy Letby?

"Answer: I think that's possible. I've tried to explain in my statement here that those are my recollections.

"Question: All right.

"Answer: It's difficult to remember exactly --

"Question: Yes.

"Answer: -- when you're going back over a period of years.

"Question: Yes."

MR JOHNSON: That's all absolutely right. That's why the text is so important, which I've just dealt with with the witness because she is --

MR JUSTICE GOSS: The point is that he believed it was the defendant, he gave evidence, but accepted it is possible that it was Shelley Tomlins. I'm looking at my notes and I've picked up these very same points in my notes.

So there we are.

I think it shouldn't be suggested that unequivocally, to a standard of sureness, for example, that you're saying --

MR JOHNSON: I don't think that's what I said, I said:

"Did you forget the evidence of Dr Neame?"

MR JUSTICE GOSS: Anyway, be that as it may, you may rest assured when I come to remind the jury of the evidence of Dr Neame on this, I will represent fairly and accurately what the totality of his evidence was.

MR MYERS: We're grateful, my Lord. I leave the matter with my learned friend and we will deal with it as we see fit if necessary in due course.

MR JUSTICE GOSS: Let's leave that for now. Thank you very much.

(In the presence of the jury)

MR JUSTICE GOSS: Sorry that was rather than longer than expected.

Mr Johnson.

MR JOHNSON: Thank you, my Lord.

Right. We'd just dealt with the neonatal review concerning events preceding the first collapse of [Baby H] on the night shift of the 26th into 27 September.

LETBY: Yes.

JOHNSON: And we had established that on the face of the document, at page 4 of 11, you had had no connection whatsoever with [Baby H].

LETBY: Yes.

JOHNSON: And we had also established that the text you sent to your friend [Nurse A] at 21.51, which is tile 406, is you telling her that you were helping Shelley.

LETBY: Yes.

JOHNSON: And my question was to what were you referring? Can you tell us?

LETBY: Sorry?

JOHNSON: What were you talking about that you had been helping Shelley with?

LETBY: I think from that it reads that I have been helping Shelley with [Baby H] and I've been involved but I haven't got the responsibility of her this evening.

JOHNSON: Was that your opportunity to sabotage [Baby H] because you were not connected with her on the paperwork?

LETBY: No.

JOHNSON: Go on to tile 405, please. That's just back a few minutes. What we see there is Shelley Tomlins' recording of the desaturation of [Baby H]. She times it there as 21.45; can you see that?

LETBY: Where is it, sorry?

JOHNSON: If you go to the -- sorry, this is a note she made at 22.53. So it says:

"Around 20.30, profound desat and brady."

LETBY: Yes.

JOHNSON: "Air no longer heard."

I think it then -- if you go down a bit, this is the one I was just talking about:

"21.45. Desat to 40% despite good entry and positive capnography. ETT suctioned quickly with thick bloodstained secretions noted. Have you got that?

LETBY: Yes, thank you.

JOHNSON: So this is, as was the earlier desaturation, according to what Shelley Tomlins was noting, both desaturations were associated with bloodstained secretions coming from the tube, the ET tube.

LETBY: Um... At 21.45, yes.

JOHNSON: It is.

LETBY: I haven't looked at the 20.30.

JOHNSON: You see where it says "CP" towards the left of the thick black line?

LETBY: Yes.

JOHNSON: There's five CPs. If you go to the fourth, you'll see:

"Copious secretions obtained via ETT and orally bloodstained."

LETBY: Yes.

JOHNSON: So both blood coming up from the lungs, as you told us, through the ET tube if it's in the right place --

LETBY: Yes.

JOHNSON: -- but also blood orally, blood in the mouth?

LETBY: Yes.

JOHNSON: Like [Baby E], isn't it?

LETBY: I can't comment on that. That's what Shelley saw.

JOHNSON: Well, that's what you saw with [Baby E], isn't it? Blood in the mouth? Vomit of blood?

LETBY: Yes, that's different to finding bloodstained mucousy secretions in the oral cavity. This isn't an active bleed.

JOHNSON: You'd been interfering with [Baby H]'s tube, hadn't you?

LETBY: No.

JOHNSON: And that's why she was desaturating?

LETBY: No.

JOHNSON: You then got involved more heavily with [Baby H], didn't you?

LETBY: I can't recall. I think I did help at some point, yes.

JOHNSON: Yes. Let's have a look. There are two ways of doing it. We can do it both ways at the same time. If you turn over the neonatal review to page 5 of 11, this may be the quickest way of doing it, and if you want further detail, please say so, and we'll go to the sequence of events, all right --

LETBY: Okay.

JOHNSON: -- which has the more detailed data? Lines 71 to 74, 22.12 to 22.13, giving medication.

LETBY: Yes.

JOHNSON: So that in effect is a double-up, isn't it? It's two events, but recorded separately in each case?

LETBY: That's right.

JOHNSON: In two separate places?

LETBY: Yes.

JOHNSON: At lines 82 to 83 at 22.38, again a single event, giving medication to [Baby H]?

LETBY: Yes.

JOHNSON: If we turn over the page just to keep the chronology clear, we then have a further and perhaps more serious event concerning [Baby H] at line 115.

LETBY: Yes.

JOHNSON: Have you got that?

LETBY: Yes.

JOHNSON: I just want to deal with what you were doing at about that time, okay --

LETBY: Okay.

JOHNSON: -- just before midnight. Were you bored with the children that you were looking after in nursery 2?

LETBY: No.

JOHNSON: Did you have time on your hands?

LETBY: No.

JOHNSON: Let's look at tile 433 in the sequence of events. This is at 00.45. To a dinosaur like me who doesn't understand Facebook, what does that mean?

LETBY: That I've liked that person's post.

JOHNSON: Who is Elizabeth Jane?

LETBY: I'm not sure. I don't recall who that is now.

JOHNSON: So someone that you don't even really know very well.

Could we summarise it that way then if you don't remember who they are?

LETBY: Yes.

JOHNSON: You're, what, looking at their Facebook account?

LETBY: Yes.

JOHNSON: On your phone?

LETBY: Yes.

JOHNSON: And this involves some positive action by you to like whatever it is that Elizabeth Jane had posted?

LETBY: Yes.

JOHNSON: Tile 434, please. A minute later, scrolling through -- Janet Cox was one of the band 4s at the time, wasn't she?

LETBY: Yes.

JOHNSON: Scrolling through her photos on her Facebook page?

LETBY: Yes.

JOHNSON: And liking those?

LETBY: Yes.

JOHNSON: Were you a bit bored at quarter to 1 that morning?

LETBY: I can't say. I may have been on my break at this point.

JOHNSON: You may have been on your break?

LETBY: Yes.

JOHNSON: Well, we know, if we look at the neonatal review at line 121, you were doing observations on the chart for the child EM.

LETBY: Yes, at 1 o'clock.

JOHNSON: Yes. I'm going to remind you of what [Baby H]'s dad, [Father of Baby H], said. Okay?

LETBY: Yes.

JOHNSON: And again this is agreed evidence. This day -- this is -- the 26th, was a Sunday, wasn't it, going into -- sorry, a Saturday going into a Sunday, the 26th into the 27th?

LETBY: Um, yes.

JOHNSON: All right. It's almost impossible to remember these things. So that you can be sure that I'm not misleading you, let's go back to the sequence of events, please, to tiles 429 and then 430, and we'll see this fact recorded.

429, Saturday, 26 September, and 430, Sunday, 27 September.

LETBY: Yes.

JOHNSON: So this is the evidence of [Father of Baby H] again, the agreed evidence. What he said to the jury through his statement was this, talking about his daughter:

"She was okay during the Saturday day and then quite late on, I remember going to the parents' bedroom on the ward to get some sleep. It was only shortly afterwards that we got a knock on the door to say she had deteriorated and she needed medical attention and for us to go and be with her at her bedside."

Okay?

LETBY: Yes.

JOHNSON: So yet again, with [Baby H], a second incident happening just after the parents had left -- or the parent in this case --

LETBY: Yes.

JOHNSON: -- had left. [Father of Baby H]'s leaving gave you the opportunity to sabotage [Baby H], didn't it?

LETBY: No.

JOHNSON: Just as in the cases of [Baby B], [Baby C], [Baby E] and, we will see, [Baby I], [Baby N], [Baby M] and [Babies O & P]. All cases where the children deteriorate shortly after their parents leave. Is that something that you identified as an opportunity to attack these children?

LETBY: No, I've never attacked any child.

JOHNSON: Were you covering for Shelley Tomlins? You said you might have been on a break, but were you covering for Shelley at 1 o'clock?

LETBY: I couldn't say. There's no way from memory that I would know.

JOHNSON: Well, if we go to tile 425, please. Just look at the observation chart there. This is the Saturday into the Sunday, the 26th into the 27th. If we look, we can see that [Baby H] was on hourly observations, wasn't she?

LETBY: Yes.

JOHNSON: But there's no hourly observations at 01.00 hours, are there?

LETBY: No.

JOHNSON: And we know the precise time of the crash call because if we go to tiles 438 and 439, please, we can see that -- if we can click on them just to remind ourselves. 438 was the call to the consultant Dr Saladi at 01.04. The next one, 439, at 01.06, was for the SHO.

LETBY: Yes.

JOHNSON: This shows us, doesn't it, what time the emergency occurred?

LETBY: Yes.

JOHNSON: And again, Dr Neame, I'm suggesting, told us that when he arrived, the person he remembered as being there was you; do you remember that?

LETBY: I remember that, yes.

JOHNSON: Was he telling the truth?

LETBY: I don't recall that being the case.

MR MYERS: Back again to this each time.

MR JOHNSON: Well, is that right?

MR JUSTICE GOSS: "Were you there at that time?" That's the question.

LETBY: I can't say from memory.

MR JOHNSON: He said he was concerned about this because it wasn't completely clear why [Baby H] had collapsed. You were there, weren't you?

LETBY: I can't say from memory exactly what I was doing when.

JOHNSON: Well, you've made an entry as a matter of fact, if you look at the neonatal review at line 121, in the chart of the child EM.

LETBY: Yes.

JOHNSON: And that was to give yourself an alibi, wasn't it, for your interference with [Baby H]?

LETBY: No, that's me giving care to the baby that I was allocated.

JOHNSON: Do you remember [Baby H] having CPR and being given adrenaline again?

LETBY: Yes.

JOHNSON: Do you remember later, at 3.30, again [Baby H] required Neopuffing?

LETBY: I can't recall specific times, no.

JOHNSON: Let's go to tile 471, please. This is Shelley Tomlins' note of this event. Can you see where it says "addendum" about a third of the way down? So this is written at 3.49; do you see?

LETBY: Yes.

JOHNSON: This concerns an event which, if her times are correct, happened 19 minutes earlier; do you see that?

LETBY: Yes.

JOHNSON: It says:

"03.30. Profound desaturation to 60s, again requiring Neopuffing with no known cause for desat. Air entry still heard bilaterally."

So that means, doesn't it, air is going in and out of the lungs?

LETBY: Yes.

JOHNSON: "Capnography positive", which means the gas exchange is happening?

LETBY: Yes.

JOHNSON: CO2 is coming out. Then:

"Copious amounts of secretions which were pink-tinged."

LETBY: Yes.

JOHNSON: Had you interfered with [Baby H]'s ET tube again?

LETBY: No.

JOHNSON: But again, if we go to the neonatal review at page 8 of 11, there you were back in nursery 1 again, weren't you?

LETBY: When, sorry?

JOHNSON: Just after this incident which Shelley Tomlins records as being at -- I think it was 03.30, wasn't it?

LETBY: Yes, I'm helping Shelley after that, yes.

JOHNSON: Mm. The supernumerary on this shift was Christopher Booth, wasn't he?

LETBY: He was the shift leader, yes. I don't think he was supernumerary.

JOHNSON: Well, he had one child in nursery 2, didn't he?

LETBY: Yes.

JOHNSON: Which is where you were.

LETBY: Yes.

JOHNSON: Why is it always you that ends up back in nursery 1 when something happens?

LETBY: I don't agree that it is always me.

JOHNSON: You tried to kill [Baby H] twice, didn't you?

LETBY: No.

JOHNSON: I'm going to move on to [Baby I] next, please. If we go back to jury bundle 1, divider 1, we can see that the single count concerning [Baby I] is count 12 at page 5.

Have you got that?

LETBY: Yes.

JOHNSON: And you remember [Baby I] very well, don't you?

LETBY: Yes.

JOHNSON: Hers was a case in which, again, I'm going to suggest you falsified the records; okay?

LETBY: Okay.

JOHNSON: Before we get to that, though, I just want to remind you of where [Baby I] was and when; all right?

LETBY: Yes.

JOHNSON: [Baby I] was born on Friday, 7 August 2015 at Liverpool, Liverpool Women's Hospital. She remained there until 18 August when she moved to Chester, which was a Tuesday.

She remained at Chester from 18 August to 6 September, which was a Sunday, when she went back to Liverpool. A week later on Sunday, 13 September, she moved from Liverpool back to Chester. Then, 17 days later, on 30 September, she collapsed in the circumstances which we've referred to as event 1 for [Baby I] and for which there is a sequence of events, which is numbered [Baby I] 1. All right?

LETBY: Yes.

JOHNSON: The second collapse for [Baby I] was 13 October and that was the incident in nursery 2 involving you and Ashleigh Hudson.

LETBY: Yes.

JOHNSON: The third was the following night, 14 October, following which she was sent out from Chester to Arrowe Park the next day, on 15 October. She remained there until 17 October when she was returned to Chester; right?

LETBY: Okay.

JOHNSON: Then 6 days later, [Baby I] died and we suggest it was you who killed her; all right?

I'd like to start with your defence statement, please, if you go to that document again. The section concerning [Baby I] starts at 106, where you say you did nothing to harm her. At 107, you say her stomach was bloated a lot of the time, it swelled regularly and she was monitored because of that.

This is what you write:

"This appeared to be part of her condition throughout her time on the unit and all the nursing staff knew that. She'd been to Liverpool Women's Hospital and Alder Hey and they didn't appear to do anything about it."

I think in this context, Alder Hey may be a mistake; is that right?

LETBY: No, she did go to Alder Hey.

JOHNSON: She did go to Alder Hey?

LETBY: Yes.

JOHNSON: When did she go to Alder Hey?

LETBY: I can't be specific on time and dates.

JOHNSON: You then say:

"I don't know whether tests were ever done to identify what the cause of [Baby I]'s problems was and whether there was something wrong with her bowel. The nursing staff raised concerns about [Baby I]'s stomach but nothing was ever done about this."

Are you one of the people that raised concerns?

LETBY: Yes.

JOHNSON: To whom did you raise the concern?

LETBY: To senior nursing staff.

JOHNSON: Who?

LETBY: I can't be specific on names.

JOHNSON: How did you raise the concern?

LETBY: It was discussed regularly amongst the staff on the unit that we were not getting anywhere with [Baby I], that she was not getting the treatment that she perhaps needed.

JOHNSON: Did you see that as an opportunity?

LETBY: No.

JOHNSON: At 109:

"[Baby I] was a baby who became unwell regularly and who deteriorated very quickly when she did so. It appeared that she often followed a pattern of desaturation and apnoea, followed by intervention. I didn't look after [Baby I] a great deal and many of the incidents throughout her history occurred when I was not present. I don't recall any incident when I said, 'It's always me when it happens', and I don't know who Lucy Beebe is."

Dr Beebe, Lucy Hunt, did you remember her when she came to court?

LETBY: No.

JOHNSON: You say you were not working on the 22nd or 23 August.

You say you don't remember much detail of 30 September, which is her first collapse. And you then say:

"I do recall that when I handed over to Bernadette Butterworth, [Baby I] desaturated and became bradycardic. Her tummy became bigger as we watched. We aspirated her manually. I then assisted with her care. I would have written my notes up and gone home, having done this."

Was that the limit of what you remembered of the first [Baby I] incident?

LETBY: Yes. I don't remember any great detail, no. But I do remember the handover with Bernadette.

JOHNSON: Right. Well, to put this into context, event number 1 happened on, as I've already said, 30 September. There were two events in effect: the first at 16.30, when Bernadette Butterworth wouldn't have been around, when there was a large vomit and desaturation.

LETBY: Yes.

JOHNSON: Then a further desaturation at handover at about 19.30.

LETBY: Yes.

JOHNSON: Do you remember that?

LETBY: Yes.

JOHNSON: Do you remember both incidents?

LETBY: Yes.

JOHNSON: Is it your case that staffing levels caused or contributed to [Baby I]'s problems on this occasion?

LETBY: No.

JOHNSON: Just to be fair to you, I hope, can we go to tile 44?

(12.33 pm)

(No video feed from court)

(12.35 pm)

MR JOHNSON: I don't think the transcriber caught any of the population distribution, so I'll run through it a bit more quickly. You had three children, including [Baby I], in nursery 3?

LETBY: Yes.

JOHNSON: Ashleigh, two in 1. [Nurse B] and Melanie Taylor had, between them, three in 2 and the nursery nurse Jean Peers had three in nursery 4?

LETBY: Yes.

JOHNSON: So staffing levels not an issue; is that right?

LETBY: Not for [Baby I], no.

JOHNSON: Medical competence or incompetence an issue?

LETBY: No.

JOHNSON: Anybody make a mistake that you have been able to identify that may have contributed to what happened to [Baby I] on this occasion?

LETBY: No.

JOHNSON: The day before, so 29 September, was a Tuesday. Okay?

And Dr Beebe told us that she had examined [Baby I] in preparation for the grand round; okay?

LETBY: Yes.

JOHNSON: She found her to be in good condition, with no markers of concern.

LETBY: No.

JOHNSON: And when you took over on the morning of Wednesday the 30th, did you see anything to contradict that opinion?

LETBY: No. From my memory now, I think there maybe was some concern about her temperature, but I don't recall any significant concerns, no.

JOHNSON: Right. Well, as you've raised it, let's deal with it.

If we go in the paper bundle to divider 12. If we start on the first page, just to give us the context. This is page J14714. That shows us [Baby I]'s temperature on the 26th, 27th, 28th and 29 September, doesn't it?

LETBY: Yes.

JOHNSON: We see that she just strays into the yellow area at about 01.00 hours and just before on the 28th?

LETBY: Yes.

JOHNSON: Moving on to the day I've just asked you about with Dr Beebe and preparation for the grand round, everything's within the white area --

LETBY: Yes.

JOHNSON: -- so far as temperature is concerned?

LETBY: Yes.

JOHNSON: And the first entry that you made appears to be at 10.00 hours --

LETBY: Yes.

JOHNSON: -- on the morning of the 30th.

LETBY: Yes.

JOHNSON: Agreed so far. Do we also see that other than having her temperature taken every 3 hours or so, and the cot temperature being recorded or the incubator temperature being recorded, no other data was being recorded for [Baby I]?

LETBY: That's right.

JOHNSON: As a general rule, does that indicate that there were no specific concerns concerning [Baby I]?

LETBY: No, that's right.

JOHNSON: The first aberration, if I can use that phrase, is recorded by you in the temperature column at -- is that 12 or 13.00 hours?

LETBY: It's 11.00.

JOHNSON: Sorry, 11.00. Have you written there 36 degrees?

LETBY: 36.1.

JOHNSON: 36.1. So the temperature of the -- is it an incubator or a cot?

LETBY: A hot cot.

JOHNSON: That's been increased to 37?

LETBY: Yes.

JOHNSON: And we see your initials at the bottom?

LETBY: Yes.

JOHNSON: Right so far?

LETBY: Yes.

JOHNSON: As a matter of fact, you didn't like being in nursery 3, did you?

LETBY: I liked being in all of the nurseries.

JOHNSON: And you didn't like Ashleigh Hudson because she was the new girl?

LETBY: I don't agree with that.

JOHNSON: By this stage you didn't like Mel Taylor either because she wouldn't talk to you in the aftermath of [Babies A & B]?

LETBY: No, I don't agree.

JOHNSON: Or [Nurse B]?

LETBY: No.

JOHNSON: Tile 46, please. This is Dr Newby's note from the grand round. If we just scroll gently down so that you can re-familiarise yourself with its contents. Most of the writing may well be Dr Beebe's writing I think she told us. A lot of this information was recorded the previous day, wasn't it, in preparation for the grand round?

LETBY: Yes, that's what usually happens, yes.

JOHNSON: Yes. If we continue down the data, please, Mr Murphy, on to the next page, right to the bottom, please. There we see - you see where it says "management plan" and there's a flamboyant signature of Dr Hunt, as she then was, now Dr Beebe?

LETBY: Yes.

JOHNSON: Followed by Dr Newby's handwriting in the box at the bottom of the page?

LETBY: Yes.

JOHNSON: The 30th was a Wednesday, so immunisations the following Monday, that was the plan?

LETBY: Yes.

JOHNSON: Establishing feeds?

LETBY: Yes.

JOHNSON: "Wean hot cot as appropriate", so get her off the hot cot as her temperature stabilises?

LETBY: Yes.

JOHNSON: "Feed and grow"?

LETBY: Yes.

JOHNSON: A positive picture; do you agree?

LETBY: Yes.

JOHNSON: You, I'm suggesting, will remember that [Baby G] on 7 September, so in round terms 3 weeks and 2 days before this, was due her immunisations.

LETBY: Yes.

JOHNSON: And the event that precipitated the immunisations being delayed was a projectile vomit, wasn't it?

LETBY: Yes.

JOHNSON: Can you remember what the event that precipitated the delay of [Baby I]'s immunisations was?

LETBY: Her condition on this day.

JOHNSON: Yes, which manifested itself in what way?

LETBY: She vomited and needed to be transferred into nursery 1.

JOHNSON: Yes. Can we look at tile 48, please? Here we see the feeding chart for this day, don't we?

LETBY: Yes.

JOHNSON: Overnight your friend -- is that J Jones, Jennifer Jones-Key?

LETBY: Yes.

JOHNSON: Gave a bottle to [Baby I] at 00.30 --

LETBY: Yes.

JOHNSON: -- of 35ml?

"Passed urine. Bowels not opened."

NB, what's that?

LETBY: That the baby has two name bands in situ.

JOHNSON: Then comment:

"Distended" --

LETBY: No, it's "demanded", so she cried for the feed and had cares.

JOHNSON: So very positive signs?

LETBY: Yes.

JOHNSON: This is a baby who was doing well, who was demanding milk and taking it from a bottle at 00.30?

LETBY: Yes.

JOHNSON: Given a further feed 3 hours later?

LETBY: Yes.

JOHNSON: And it may or may not be NGT, but we can move on to 07.00 hours, a further bottle?

LETBY: Yes.

JOHNSON: Passed urine?

LETBY: Yes.

JOHNSON: Bowels opened?

LETBY: Yes.

JOHNSON: Still got the name bands. And given cares.

LETBY: Yes.

JOHNSON: Incidentally, how often were baby given cares?

LETBY: Usually with every feed, so a baby that's on three-hourly feeds like [Baby I] was, it would be done with each feed.

JOHNSON: What about a baby in nursery 1?

LETBY: Cares are done less often then. It depends on the baby.

JOHNSON: But minimal disruption in nursery 1 is the objective?

LETBY: For babies that are intensive care in nursery 1, yes.

JOHNSON: Yes, okay. Let's just carry on. So that was just before your friend Jennifer Jones-Key handed over to you at about 8 am?

LETBY: Yes.

JOHNSON: You have then recorded at 10.00 hours having given [Baby I] expressed breast milk with fortifier?

LETBY: That her mum fed her, but yes.

JOHNSON: You're quite right. 35ml?

LETBY: Yes.

JOHNSON: "Passed urine and small bowels opened", is that?

LETBY: Yes.

JOHNSON: [Baby I]'s mum fed her and did her cares?

LETBY: That's right.

JOHNSON: At 13.00 hours, and we do have this in paper as well of course, we have a further feed, this time given by you?

LETBY: Yes.

JOHNSON: Given by NG tube?

LETBY: Yes.

JOHNSON: With an aspirate of 5ml; is that right?

LETBY: Yes, that was returned.

JOHNSON: Which was returned, with a pH of 5?

LETBY: Yes.

JOHNSON: Did that cause you any concerns?

LETBY: No.

JOHNSON: No concerns about an undigested feed?

LETBY: No, 5ml is a very minimal amount.

JOHNSON: Yes. A pH of 5 is an acidic reaction, isn't it?

LETBY: Yes.

JOHNSON: Was [Baby I] asleep?

LETBY: Yes.

JOHNSON: How she was according to you at 16.00 as well?

LETBY: Yes.

JOHNSON: Was she a particularly sleepy baby, [Baby I]?

LETBY: I don't think I can comment on [Baby I] on a day-to-day basis. At this time we changed her nappy, or I changed her nappy, and she did not wake.

JOHNSON: Again, you fed her via the NG tube?

LETBY: Yes.

JOHNSON: A 3ml aspirate, again nothing of concern?

LETBY: No.

JOHNSON: Passed urine?

LETBY: Yes.

JOHNSON: Bowels opened?

LETBY: Yes.

JOHNSON: Do we take it that her stomach was empty at that stage?

LETBY: Prior to the feed?

JOHNSON: Yes.

LETBY: She had 3ml, yes.

JOHNSON: And then half an hour later:

"Large vomit, plus apnoea. [Is that] Sent to nursery 1..."?

LETBY: Yes.

JOHNSON: "... and put nil by mouth"?

LETBY: Yes.

JOHNSON: Okay. So just going back to the 10 o'clock entry and the presence of [Mother of Baby I], who was [Baby I]'s mum, do you remember what her routine was?

LETBY: Not specifically. I know that she visited very regularly for [Baby I], but I couldn't give you any more detail than that.

JOHNSON: I think one of the points you made when you were being asked about the card that you sent to the [Family of Baby I] on the day of the funeral in November was that you got to know the family very well.

LETBY: Yes. The family were very attentive and present on the unit a lot, yes.

JOHNSON: And they had other children?

LETBY: They did, yes, which they often brought as well.

JOHNSON: Yes, and [Mother of Baby I], when she dropped the kids off at school, would come straight to see her new daughter, wouldn't she?

LETBY: I know she visited regularly, yes.

JOHNSON: And would also disappear at certain times to go and collect her children from school?

LETBY: Potentially, yes.

JOHNSON: When you say potentially --

LETBY: I can't account for exactly what [Mother of Baby I] did when.

JOHNSON: I'm suggesting you knew this family so well that you know very well that that's what her routine was.

LETBY: No, I don't agree.

JOHNSON: Tile 51, please. Could we look at the review? We see Dr Beebe's writing again. Can you see that?

LETBY: Yes.

JOHNSON: What was the reason that you gave to Dr Beebe for this review?

LETBY: [Baby I] had a low temperature.

JOHNSON: "In hot cot. Temperature 36.2."

Slightly on the low side?

LETBY: Yes, that is low.

JOHNSON: "Taking full bottles and gaining weight"?

LETBY: Yes.

JOHNSON: A positive sign?

LETBY: Yes.

JOHNSON: "On examination, little pale, handles well. Capillary refill time less than 2 seconds. Warm, well perfused.

Normal heart sounds. Chest clear. No distress"?

LETBY: Yes.

JOHNSON: "Abdomen full but soft. Reducible umbilical hernia"; is that right?

LETBY: Yes.

JOHNSON: Then underneath that, what does it say?

LETBY: "During examination, bowels open, normal yellow seedy stool. Mum reports low temperature has been happening over past few days."

JOHNSON: Yes. Then:

"Plan: increase temperature of hot cot."

LETBY: "Monitor closely. If further concerns, for sepsis screen but appears clinically well at present."

JOHNSON: Yes. Do you remember Dr Beebe's evidence about that note? She was asked specifically about it.

LETBY: No, I can't, no.

JOHNSON: She said that that should read:

"Monitor closely, [if] further concerns, for sepsis screen but appears clinically well at present."

LETBY: Okay.

JOHNSON: Do you remember her saying that to you?

LETBY: No, I don't, no. What, at the time?

JOHNSON: Yes.

LETBY: No, I don't recall this conversation at that time, no.

JOHNSON: But it's clear that the reason you were giving to Dr Beebe for the review was the reduced temperature, wasn't it?

LETBY: Yes, which is correct procedure.

JOHNSON: Yes. Do you remember the reason you gave to [Mother of Baby I]?

LETBY: Not for this morning review, no. I know later on when the doctors were called it was in regards to her stomach.

JOHNSON: Well, you told [Mother of Baby I] that you were concerned about [Baby I]'s stomach. That's in her agreed statement.

LETBY: Okay. At 11 o'clock?

JOHNSON: Yes, the review.

LETBY: Okay.

JOHNSON: Any reason why you give one reason to the mother and a different reason to the doctor?

LETBY: No, I don't recall that. I recall it being an abdominal issue at the further review in the afternoon.

JOHNSON: Well, just so that you know where I'm going, I'm going to suggest to you that there was no further review in the afternoon. Okay? So you bear that in mind whilst I'm asking you these questions.

Do you remember the evidence of Dr Ogden --

LETBY: No.

JOHNSON: -- who was read on 26 January? Dr Ogden said if ever a concern was expressed about a swollen abdomen, the medical staff would always examine the baby.

LETBY: Yes.

JOHNSON: Does that accord with your understanding of what the practice was at Chester?

LETBY: Yes, but not for [Baby I]. [Baby I] had a regularly distended abdomen.

JOHNSON: My question for you to answer, please, is: why were you telling [Mother of Baby I] that the reason for your concern was [Baby I]'s abdomen but you didn't mention that to Dr Beebe?

LETBY: I don't believe I did mention the abdomen at that point to [Mother of Baby I].

JOHNSON: Well, that was the agreed evidence in the statement that was read to the jury. You're disputing that?

LETBY: Yes.

JOHNSON: Were you creating a cover for what you planned to do to [Baby I]?

LETBY: No.

JOHNSON: [Baby I] was handling well, wasn't she?

LETBY: Yes.

JOHNSON: And during this examination she produced a normal yellow seedy stool?

LETBY: Yes.

JOHNSON: Quite a graphic description. Her chest was clear; is that right?

LETBY: Yes.

JOHNSON: She was in no distress?

LETBY: No.

JOHNSON: And Dr Beebe told you to monitor her closely?

LETBY: Yes.

JOHNSON: Why didn't you monitor her closely?

LETBY: I did monitor her closely.

JOHNSON: Well, let's look at the vital signs chart, please. It's in that paper bundle that you've got there in front of you at page 14715.

LETBY: Yes.

JOHNSON: If we can scroll up a bit, Mr Murphy, so we can just remind ourselves of the time of this: 11.40.

When is the first time you recorded [Baby I]'s vital signs?

LETBY: As in a full set of observations?

JOHNSON: Yes, monitoring her closely, yes.

LETBY: No, I -- that's taken out of context.

JOHNSON: You tell us what the context is then. Well, answer the question and then tell us what the context is. When was the first time you monitored [Baby I]'s vital signs?

LETBY: 15.00.

JOHNSON: What's the context of monitor closely?

LETBY: It's with regards to the temperature. At this point [Baby I] was not on a monitor and was not deemed by the doctor that she needed to go back on a monitor. So it's monitor closely her temperature and just [Baby I] in general.

JOHNSON: All right. Let's scroll down again and look at Dr Beebe's note, please. Where does it say "monitor temperature closely"?

LETBY: The plan above is to increase temperature of hot cot and that is why [Baby I] was being reviewed because of a temperature issue.

JOHNSON: Not according to [Mother of Baby I].

LETBY: Okay.

JOHNSON: But that aside, where does it say "monitor temperature closely"?

LETBY: It doesn't. She also doesn't ask me to put [Baby I] back on to a monitor.

JOHNSON: Well, some people might think, and I'm suggesting to you that you knew, that "monitor closely" meant precisely that.

LETBY: No, I disagree.

JOHNSON: Let's go back to the feeding chart, please, which we were looking at before. It's page 14780 in the paper bundle.

We've seen that [Baby I] had been an enthusiastic feeder before you came on duty.

LETBY: Yes, she'd waken for a feed at 12.30, yes.

JOHNSON: She had taken a bottle at 7 o'clock?

LETBY: Yes.

JOHNSON: And she had taken a bottle from her mum at 10 o'clock?

LETBY: Yes.

JOHNSON: But at 1 o'clock, you were giving her milk down an NGT?

LETBY: Yes.

JOHNSON: Do you remember where [Mother of Baby I] was at 1 o'clock?

LETBY: No.

JOHNSON: Again, in her agreed evidence she said that at lunchtime she would meet her family in the canteen.

LETBY: Okay.

JOHNSON: Was [Mother of Baby I] there when you gave this nasogastric tube to [Baby I]?

LETBY: I can't say, I don't recall.

JOHNSON: If you were giving a 35ml feed with thickener at 13.00 hours that would take, what, about 15 minutes?

LETBY: Potentially, yes. I can't ever be specific, but yes, around that time, yes.

JOHNSON: So it would have ended, if that's right, at about quarter past 1?

LETBY: Roughly, yes. So the feeds would not be on the exact hour necessarily, but it would have been around 13.00, yes.

JOHNSON: Can we go to tile 61, please? Keep the feeding chart open, please, ladies and gentlemen, if you've got it in paper, which is 14780.

Let's just look at what you were writing down on the nursing note, can we? This is a nursing note made at what time?

LETBY: 13.36.

JOHNSON: Yes. What time did you give the feed?

LETBY: 13.00.

JOHNSON: Right. And a feed which normally would have finished at about 13.15; correct?

LETBY: Approximately, yes.

JOHNSON: Yes, obviously give or take. So it would have been fresh in your mind when you're making this nursing note a few minutes later, wouldn't it?

LETBY: Yes.

JOHNSON: And what did you write about feeding?

LETBY: "3x8 feeds EBM: two bottles to 1 NG tube."

JOHNSON: What is do that mean?

LETBY: That she's had two bottles to one NG feed. So usually we would give two bottles in a row then an NG tube feed and then two bottles and an NG.

JOHNSON: Right. Then what does it say?

LETBY: "Abdomen appears full and slightly distended, soft to touch. [Baby I] straining ++."

JOHNSON: And?

LETBY: "Bowels have been opened. Mum feels it is more distended to yesterday and that [Baby I] is quiet. Appears generally pale. Not on monitor but nil increased work of breathing. Handling well and waking for feeds. Doctors asked to review. To continue with current plan."

JOHNSON: Do you remember [Mother of Baby I]'s evidence that it was you who was raising the issue about [Baby I]'s abdominal distension, not her?

LETBY: I agree, yes, I raised concerns about her distension, yes.

JOHNSON: That wasn't the reason for calling Dr Beebe though, was it?

LETBY: No. That was in regards to the low temperature.

JOHNSON: "Doctors asked to review", something we've seen before.

What does that mean?

LETBY: That doctors were asked to review [Baby I] and "to continue with current plan", which is what Dr Hunt advised.

JOHNSON: No, it wasn't what Dr Hunt advised at all, was it, at 11.40? Dr Hunt had advised, "Monitor closely".

LETBY: We monitor all of our babies closely.

JOHNSON: Well, if that's true why on earth would Dr Hunt say to do something that you always do, Nurse Letby?

LETBY: That's something to ask her.

JOHNSON: Well, you're lying about this, aren't you?

LETBY: No.

JOHNSON: This is yet another example of you not reproducing in the nursing notes what really happened?

LETBY: No, I don't agree.

JOHNSON: "Handling well and waking for feeds."

What had you just done?

LETBY: Fed her because she was asleep.

JOHNSON: Yes. Well, I'm obviously a bit slow. Explain that to me.

LETBY: I'm referring to 10 o'clock. She's woken for the feed and handled well.

JOHNSON: What you're writing down is at 13.36, isn't it?

LETBY: Yes.

JOHNSON: The feed you gave, if it started bang on 13.00 hours, had finished more or less 20 minutes before this.

LETBY: Yes.

JOHNSON: Why did you write, "Handling well and waking for feeds", if this was a child to whom you had just given an NGT --

LETBY: Because the feeding regime is two bottles to one NG tube, so the bottle that she was due for from me at 10 o'clock she did wake for and was handling well. The plan then would have been for the 13.00 feed to be a tube feed.

JOHNSON: Can I just check if this note continues, Mr Murphy?

That's the end of it, okay.

Tile 62, please, just if we just deal with this point, please, my Lord, because we are going past 1 o'clock.

Can we go to tile 62? It's an addendum note made much later at 19.31. This says:

"Reviewed by doctors [again plural] at 15.00 as [Baby I] appeared mottled in colour with distended abdomen and more prominent veins. Advised to continue. Temperature within normal range with hot cot at 38 degrees. Full monitoring recommenced."

I'm going to suggest to you that this is the advice or the direction Dr Beebe was giving you at 11.40; okay?

LETBY: No, I disagree.

JOHNSON: "Full monitoring recommenced. Within normal range.

Bowels opened."

Then what does it say?

LETBY: "Yellow seedy stool."

JOHNSON: Yes. Let's just go back to Dr Beebe's note at 11.40, which is tile 51.

You have transposed what Dr Beebe said at 11.40 to your nursing note at 15.00 hours, haven't you?

LETBY: Yes, I'm referring that during the shift [Baby I] has had yellow seedy stool, yes.

JOHNSON: No, but between the yellow seedy stool at 11.40, you had already made a nursing note, hadn't you, just after the feed that you gave via a nasogastric tube?

LETBY: Yes.

JOHNSON: A note that you made at 13.35?

LETBY: Yes.

JOHNSON: So you are cooking the notes again, aren't you, Lucy Letby --

LETBY: No, I disagree.

JOHNSON: -- to create the impression that [Baby I] was deteriorating just before the time at which the emergency happened?

LETBY: No, she'd had a normal yellow seedy stool.

JOHNSON: Which happens to coincide with the time at which [Mother of Baby I] went to pick up her kids from school, doesn't it?

LETBY: Yes.

MR JOHNSON: Yes.

My Lord, that may be a convenient point.

MR JUSTICE GOSS: Certainly. We'll break off and resume, please, at 2.15, members of the jury. Thank you very much.

(1.06 pm)

(The short adjournment)

(2.13 pm)

MR JOHNSON: Just before the break we'd been dealing with what I suggested to you was your invention of a medical examination at 15.00 hours. Okay?

LETBY: Yes.

JOHNSON: If we can start just by looking at the paper chart, please, behind divider 12. It's the observation chart and it's J14715.

Is there any reason why the chart goes 10, 11, 12, 13, 15, first of all?

LETBY: We don't always do temperatures every hour.

JOHNSON: But you had started to do temperatures every hour at 10 o'clock?

LETBY: Yes. And it was on the way up.

JOHNSON: Yes, but -- actually, it was on the way down from 10 o'clock, but there we are. Is there any reason why you stopped doing them at 13.00?

LETBY: No.

JOHNSON: No reason, that's just your decision?

LETBY: Well, I don't recall why I didn't do it.

JOHNSON: You'd been given the direction to monitor closely by Dr Beebe at 11.40, hadn't you?

LETBY: Yes.

JOHNSON: So why didn't you do it?

LETBY: I can't explain that.

JOHNSON: You say there was a medical examination at 15.00 hours.

That's tile 65 if we can just look at your note, please.

"Reviewed by doctors", plural. Which doctors, please?

LETBY: I think it was [Dr A].

JOHNSON: [Dr A]?

LETBY: Possibly, from memory.

JOHNSON: When you say doctor, what about the other doctor or doctors?

LETBY: I think that's just -- doctors are referred to as doctors. I don't think you can read much into the -- whether it's one person or two, it's just how I would write doctors.

JOHNSON: Sometimes you write SHO.

LETBY: Yes.

JOHNSON: Sometimes you write reg or registrar.

LETBY: Yes, as does anybody on the unit.

JOHNSON: I'm not interested in anybody else, I'm interested in why at this time you didn't attribute this examination to a specific doctor.

LETBY: I can't explain that.

JOHNSON: Well, I'm going to suggest an explanation, because you didn't want it to be attributed in case anybody asked a question of the doctor.

LETBY: No, I disagree.

JOHNSON: Because there is no medical note relating to this, is there?

LETBY: No, there isn't, no.

JOHNSON: No. And the reason there is no medical note is because you have made it up.

LETBY: No, that's not right.

JOHNSON: Well, let's run with [Dr A], your friend, for a second. We'll just hold him in abeyance for a second.

You told us that the regime for [Baby I] was bottle, bottle, NGT, bottle, bottle, NGT.

LETBY: That's what I've written there, yes. Somewhere.

JOHNSON: Let's look at the feeding charts, please. They're in the same part of this paper bundle, they're a few pages later. Let's start with page 14779, which is the day before.

LETBY: Yes.

JOHNSON: What we see there, starting at 03.00, is: bottle, bottle, NGT, bottle, bottle, NGT, bottle. Then over on to the 30th, which is the day we are concerned with, the second bottle, followed by the NGT. Followed by bottle, bottle, the first of which was given by Jennifer Jones-Key, the second of which given by [Mother of Baby I]. Then two NGTs.

LETBY: Yes, and I've explained why.

JOHNSON: You've explained the first one.

LETBY: At 16.00 we start all sets of cares by doing the baby's nappy. [Baby I] was not awake whilst I was doing her nappy, therefore I have written she's asleep and she's given an NG tube feed.

JOHNSON: You didn't do the nappy, did you?

LETBY: I don't know who -- somebody's done the nappy, I can't say if it was me or mum or whoever.

JOHNSON: How can you tell whether anyone did the nappy at 13.00?

LETBY: There's a line through it, so I don't assume that anybody did do the nappy then and that would be the point of leaving her to rest for that selective NG feed.

JOHNSON: So what does it say at 16.00 in that column?

LETBY: "Asleep".

JOHNSON: Yes, but to the left.

LETBY: Oh:

"Passed urine and bowels open."

JOHNSON: Yes. So according to you, let's get this right, [Baby I] opened her bowels at -- sorry, according to Jennifer Jones-Key, opened her bowels at 7?

LETBY: Yes.

JOHNSON: According to you, opened her bowels at 10?

LETBY: Yes.

JOHNSON: According to Dr Lucy Beebe, opened her bowels with a yellow seedy stool at 11.40?

LETBY: Yes.

JOHNSON: According to your note for [Dr A], did another yellow seedy stool at 15.00?

LETBY: I don't... Can you show me that, please?

JOHNSON: Yes, your nursing note. We were looking at it. This is the one where I'm suggesting you have transposed the information from Dr Beebe's examination at 11.40 into your invented nursing note for a fictional medical examination at 15.00. Okay?

MR MYERS: She's asked if she can see it. I wonder if she could be shown the nursing note.

MR JOHNSON: I am going to show it to her, I'm just explaining what I'm doing.

It's tile 65. It's four lines down:

"Bowels opened, yellow seedy stool."

LETBY: Yes.

JOHNSON: It says, "Full monitoring recommenced". Do you remember I suggested that's what Dr Beebe told you to do at 11.40?

LETBY: Yes.

JOHNSON: So that's what you are, we suggest lyingly, recording at 15.00.

LETBY: I don't agree.

JOHNSON: Then at 16.00, [Baby I] opened her bowels again according to you.

LETBY: Yes.

JOHNSON: Yes? Can we go to tile 73, please? This is the first medical note after Dr Beebe's medical note at 11.40, isn't it?

LETBY: Yes, it is, yes.

JOHNSON: And this is your friend, [Dr A], coming to see [Baby I]?

LETBY: Yes.

JOHNSON: No mention by [Dr A] of any previous examination that he had undertaken an hour and a half earlier, is there?

LETBY: No, there isn't.

JOHNSON: No. Is [Dr A] a meticulous keeper of records?

LETBY: I don't think I can comment on what [Dr A]'s practice is.

JOHNSON: Well, when he examined patients of yours, did he tend to keep a medical note if there was a concern?

LETBY: Doctors usually do, yes, but not all the time, no.

JOHNSON: If he'd told you to put a baby on full monitoring, would you expect him to keep a note from what you knew of him?

LETBY: If he had instructed me to do something.

JOHNSON: Yes.

LETBY: If he'd specifically said start a monitor then yes.

JOHNSON: That's what you have recorded, you've just looked at it in your nursing note at 15.00.

LETBY: Yes.

JOHNSON: And that's why we suggest you are lying about the fictional examination at 15.00.

LETBY: No, I don't agree. Just because it's not written there, that's not necessarily my error.

JOHNSON: Oh no, it's [Dr A]'s error if you're telling the truth, isn't it?

LETBY: Yes.

JOHNSON: He has failed to mention or failed to record an examination of a child who was about to crash?

LETBY: Yes.

JOHNSON: He was crash called at 16.30.

LETBY: Yes.

JOHNSON: Nothing in the history there of having previously given instructions to put this child on full monitoring, is there?

LETBY: No.

JOHNSON: No. Did you put in a Datix about his failure to record that?

LETBY: No.

JOHNSON: Why not?

LETBY: I wasn't looking at the medical notes at that time, that was not my concern.

JOHNSON: But you're very keen to keep the doctors up to the mark, aren't you?

LETBY: Yes.

JOHNSON: Very keen to complain about Dr Harkness and Dr Gibbs when they'd left [Baby G] on that trolley?

LETBY: Yes.

JOHNSON: Why didn't you complain or escalate [Dr A]'s failure?

LETBY: I don't believe this was noted at the time. My priority was with [Baby I], not reading medical notes.

JOHNSON: That medical note you made was made much later in the day, wasn't it -- sorry, that nursing note you made was made much later in the day?

LETBY: Yes.

JOHNSON: 19.31 at the end of your shift?

LETBY: Yes.

JOHNSON: You would have had plenty of time to review what people had noted by that stage, wouldn't you?

LETBY: Yes.

JOHNSON: Just going back to the feeding chart then, please, at 14780, which is tile 68, you force fed [Baby I], didn't you?

LETBY: No, I did not.

JOHNSON: And that is why you gave her an NGT feed, which was out of sync with the pattern that had been previously established over 2 days?

LETBY: No. [Baby I] did not wake for that feed therefore she was tube fed, which is standard practice.

JOHNSON: She vomited 30 minutes after you had recorded giving her that feed?

LETBY: Yes.

JOHNSON: And up to that point she had shown no signs of there being anything significantly wrong with her, had she?

LETBY: No, other than the low temperature, no.

JOHNSON: You say the low temperature, but as a matter of fact, just before the vomit, if we go back to 14715 --

LETBY: Yes, it had normalised.

JOHNSON: -- her temperature had normalised, hadn't it?

LETBY: Yes.

JOHNSON: So all the signs had been positive and despite that fact she vomited?

LETBY: Yes.

JOHNSON: Just like [Baby G]?

LETBY: Yes.

JOHNSON: When you were there?

LETBY: I don't know if I was there when she vomited.

JOHNSON: Her diaphragm was splinted, wasn't it?

LETBY: I don't recall.

JOHNSON: Well, let's look at tile 78, the radiologist's report, and go to the text, please. We can see the sausages, as Professor Arthurs referred to them:

"There is splinting of the diaphragm due to bowel distension."

LETBY: Yes.

JOHNSON: And this was a child that had vomited as well, wasn't it?

LETBY: Yes.

JOHNSON: So she'd actually relieved a fair bit of the pressure on her?

LETBY: At 16.30, yes.

JOHNSON: Yes. You had pumped her with air or milk, hadn't you?

LETBY: No.

JOHNSON: Were you excited by what you did?

LETBY: No, I fed [Baby I] a normal tube feed.

JOHNSON: Why didn't you note the blood gas that was taken at 17.15?

LETBY: It's not on the blood gas chart, is that what --

JOHNSON: Well, let's look at tile 75. You see the arrow? This is --

LETBY: Yes.

JOHNSON: -- your friend --

LETBY: Friend?

JOHNSON: Yes, one of your fellow nurses has made this note.

Do you see it?

LETBY: Okay, I wouldn't class every nurse on the unit at my friend, but --

JOHNSON: Right, okay. This is Bernie Butterworth. Is she not a friend of yours?

LETBY: No, not outside of work, no.

JOHNSON: I apologise then. Nurse Butterworth noted up that 17.15 blood gas. She came on later; do you remember?

LETBY: Yes.

JOHNSON: And she found the printout from the blood gas machine on the workstation.

LETBY: Yes, on the clipboard. I believe it was on the end of the chart.

JOHNSON: Yes, and you hadn't noted it up, had you?

LETBY: No.

JOHNSON: That's why she's written on there "PP..." --

LETBY: Yes, I agree.

JOHNSON: -- "... for LL"?

LETBY: Yes.

JOHNSON: And it comes in after her later blood gas at 21.23?

LETBY: Yes.

JOHNSON: This wasn't a blood gas that found its way home with you, was it?

LETBY: No.

JOHNSON: No. Why didn't you note it?

LETBY: I can't say for definite. It's an error, but the gas was there on the baby's chart for anybody to see that needed to see it. It was not hidden.

JOHNSON: You aspirated a lot of air from [Baby I], didn't you, at this time?

LETBY: At the time of the gas?

JOHNSON: At the time that her diaphragm was splinted.

LETBY: I don't... I don't know without looking.

JOHNSON: You don't know?

LETBY: No.

JOHNSON: Let's look at your interview, if we can, please. It's in the first bundle of interviews right at the back of the bundle. It's [Baby I] 3, which is the interview of 11 June 2019 -- it's my mistake, this is slightly later in the day when [Baby I] desaturated at handover.

All right?

LETBY: Yes.

JOHNSON: So just to remind us all, Bernadette Butterworth came on to the night shift at 19.30; is that right?

LETBY: Yes.

JOHNSON: You handed over to her, Nurse Butterworth?

LETBY: Yes.

JOHNSON: And at the point of handover, [Baby I] desaturated again?

LETBY: Yes.

JOHNSON: And you extubated -- sorry, you aspirated a lot of air from [Baby I]?

LETBY: Okay. I can't see the chart, but I agree if that's what you're saying. I can't...

JOHNSON: Well, let's deal with it --

LETBY: She did have air, yes.

JOHNSON: Let's deal with it in your nursing note. It's tile 91, please.

At 19.30, [Baby I] became apnoeic. Let me find that.

It must be further down.

This is a note made at 20.26, it should be. It may be in the top right-hand --

LETBY: It's there, I can see it.

JOHNSON: I beg your pardon. Quite right. Thank you. I'm afraid these are sometimes difficult to read:

"At 19.30, [Baby I] became apnoeic, abdomen distended ++."

Do you see that?

LETBY: Yes.

JOHNSON: Then a couple of lines below that:

"Air ++ aspirated from NG tube and chest movement and observations improved."

LETBY: Yes.

JOHNSON: You forced air into the NG tube of [Baby I], didn't you?

LETBY: No, I did not.

JOHNSON: And that is the reason why a lot of air came out after she collapsed for the second time on this shift?

LETBY: No, I do not agree.

JOHNSON: After you left for the night, [Baby I] improved, didn't she?

LETBY: I can't say without looking now.

JOHNSON: Well, let's look at the observation chart then. It's the one we had open before, 14715. Heart rate comes down. Have you got it?

LETBY: Yes, thank you.

JOHNSON: So heart rate comes down?

LETBY: Yes.

JOHNSON: The temperature of the incubator comes down as well, doesn't it?

LETBY: No, the temperature increases overnight -- oh, you mean the incubator temperature comes down, yes, but [Baby I] has a high temperature, yes.

JOHNSON: That's because the incubator is so high when you've been looking after her?

LETBY: She also had a low temperature at 20.00.

JOHNSON: Yes, but you'd only just released her from your care at 20.00, hadn't you?

LETBY: Yes.

JOHNSON: And the temperatures that we have recorded before then are all recorded by you, aren't they?

LETBY: Yes.

JOHNSON: As soon as [Baby I]'s care is handed over to somebody else, the incubator temperature comes down?

LETBY: No, I don't agree. At 20.00 she still had a low temperature and that is not my set of observations.

JOHNSON: Where were you at 20.00, Lucy Letby?

LETBY: I would have been on the unit.

JOHNSON: Yes. Just having handed over care to Bernie Butterworth?

LETBY: Yes.

JOHNSON: And what time did Bernie Butterworth start?

LETBY: Well, Bernie Butterworth was present at 19.30 when she had the episode.

JOHNSON: She was, but when did responsibility pass to Bernie Butterworth?

LETBY: Well, from that time.

JOHNSON: Which time?

LETBY: Once we start handover.

JOHNSON: Let's look at what she writes at tile 94, please. This mirrors many of your nursing notes, doesn't it, in form?

LETBY: Yes.

JOHNSON: What does it say?

LETBY: Commenced at 20.00.

JOHNSON: Yes, so you'd just literally handed over care of this child to somebody else?

LETBY: I don't agree. When we document things like that we're not documenting the exact moment in time that we handed over. 8 o'clock is the end of the day shift and the start of the night shift so usually that is when care would commence. Bernie was with me at 19.30 when I was doing handover.

JOHNSON: Well, let's move on to incident number 2 with [Baby I].

MR JUSTICE GOSS: Can we put the interviews away then?

MR JOHNSON: We can.

I just want to check that I understood what you said about this incident in answer to your counsel; okay?

Did you tell the court that you went into nursery 2 together with Ashleigh Hudson?

LETBY: Yes.

JOHNSON: And at some point when you just arrived, you noticed that [Baby I] looked pale in the cot?

LETBY: Yes.

JOHNSON: Did you say that Ashleigh was doing something on the workbench?

LETBY: Yes.

JOHNSON: You couldn't recall whether the lights were on or off but you knew that you could see clearly enough that [Baby I] was pale in the cot?

LETBY: Yes.

JOHNSON: You were standing in the doorway, you say, when you noted [Baby I]'s colour?

LETBY: Yes.

JOHNSON: Her face and her hands?

LETBY: Yes.

JOHNSON: You said to Ashleigh that she looked pale?

LETBY: Yes.

JOHNSON: You both went over and definitely turned up the lights?

LETBY: Yes.

JOHNSON: And you also said to the jury that you would be able to tell if [Baby I] was pale without the lights on?

LETBY: Yes, however the lighting was in that nursery at that time, I could see her, yes.

JOHNSON: Did you also say that you couldn't remember much outside what the notes say?

LETBY: For that particular episode, that's right.

JOHNSON: But you do remember saying that [Baby I] looked pale when you were standing at the doorway?

LETBY: Yes.

JOHNSON: And you also said that Nurse Eagles came straightaway to help?

LETBY: Yes, from my memory, Laura did attend, yes.

JOHNSON: Yes. I want to start with your defence statement, please, paragraph 115. Do you start by saying:

"I do not recall Ashleigh Hudson asking me to look after [Baby I] when she went to assist with a cannula on 13 October"?

LETBY: That's right.

JOHNSON: "[Baby I] was not due a feed at that time, she had been fed at 01.30"; yes?

LETBY: Yes.

JOHNSON: Ashleigh had been on the unit since February?

LETBY: Yes.

JOHNSON: She was quite inexperienced to be looking after [Baby I]?

LETBY: Yes.

JOHNSON: What was it about [Baby I] that posed a specific difficulty as at the beginning of the night shift on 12 October?

LETBY: So a baby like [Baby I] needs very close monitoring and being able to look for any changes that happen quite quickly. I also think, looking back, Ashleigh had stopped her monitoring when she should have been on a monitor still.

JOHNSON: All right. Unlike you, who hadn't her on a monitor even though Dr Beebe had told you to have her on a monitor?

LETBY: I disagree that Dr Beebe told me to do that.

JOHNSON: Who had told Ashleigh to be doing monitoring when she wasn't?

LETBY: No, it's the opposite: she wasn't doing monitoring when she should have been.

JOHNSON: Yes. I think that's what I said, but I'll try again, see if I can improve. Who had instructed Ashleigh to do monitoring?

LETBY: I don't understand. She wasn't monitoring [Baby I].

JOHNSON: Yes, but I thought you said that she should have been monitoring --

LETBY: Yes. She was still on antibiotics -- well, she'd been on antibiotics, so yes.

JOHNSON: Okay. Where does the instruction come from that [Baby I] should have been monitored?

LETBY: I can't see that anywhere.

JOHNSON: Well, what -- it's probably just me being thick. What makes you say that there is something that means that she should have been monitored?

LETBY: The policy is that babies remain on full monitoring for 48 hours after they've stopped antibiotics and I think that was the case for [Baby I].

JOHNSON: Right. You seem to have a good recall of this. When had [Baby I] stopped being monitored -- I'll start again.

When had [Baby I] last been given antibiotics?

LETBY: I'm not sure from my memory now.

JOHNSON: Right. Does it come to this then: because you think that possibly [Baby I] had been given antibiotics some time in the previous 48 hours, she should have still been on monitoring?

LETBY: That's the protocol, yes.

JOHNSON: Yes. Let's not worry about the protocol for the moment, let's worry about what the evidence actually is. Are you saying that, according to you -- I'm not disputing this with you at the moment, but are you saying that she had been on antibiotics --

LETBY: Yes.

JOHNSON: -- some time in the previous 48 hours?

LETBY: I think so. That's from my memory now. I can't -- I would be welcome (sic) to check the charts, but...

JOHNSON: Well, we've got plenty of time to check them, don't worry. That's your memory. Is that your memory from the time or is it a more recently acquired memory from reading the papers?

LETBY: No, it's from being able to look back retrospectively at [Baby I]'s position at that time.

JOHNSON: So are you saying that Ashleigh was too inexperienced or quite inexperienced to be looking after a child who had been on antibiotics?

LETBY: No. I'm not referring to her being on antibiotics.

[Baby I] herself was a baby that needed very close monitoring.

JOHNSON: When had she started needing this very close monitoring?

LETBY: All babies need close monitoring and [Baby I] was in nursery 2, the high dependency room.

JOHNSON: Yes, but as we've heard many times, and you have reminded me this morning, the precise location of a baby isn't necessarily determinative of the level of care required.

LETBY: No, I agree, but [Baby I] is still a baby that required close monitoring.

JOHNSON: And what does that monitoring include?

LETBY: Visually looking at [Baby I] and assessing her for any changes or any change in behaviour.

JOHNSON: Anything else?

LETBY: No.

JOHNSON: So looking at the baby, that's what's required?

LETBY: In part, yes, and seeing how the baby behaves, how the baby's tolerating feeds, if they're waking appropriately.

JOHNSON: Well, this is the middle of the night.

LETBY: Yes, so babies wake throughout the night.

JOHNSON: Yes. Well, what default, what mistake are you identifying that Ashleigh made here?

LETBY: I'm not saying that Ashleigh made a mistake, I'm saying retrospectively when I looked at the notes she should have been on a monitor and maybe that would have alerted us to her change sooner.

MR JUSTICE GOSS: Sorry, can I just be clear about this because the word monitoring is causing me some difficulty and a monitor. Right. There is a device called a monitor, an electronic monitor?

LETBY: Yes, a Masimo.

MR JUSTICE GOSS: A Masimo monitor. When you say a baby should be on a monitor, do you mean the baby should be connected to a monitor?

LETBY: Yes, to the Masimo monitor or Philips monitor, yes.

MR JUSTICE GOSS: Right, a Philips or a Masimo monitor.

What about monitoring, which means essentially keeping an eye on?

LETBY: Yes.

MR JUSTICE GOSS: Can we just be clear each time you say monitoring, which you are meaning, either on electronic monitor or monitors or just monitoring --

LETBY: I'm talking about a general picture.

MR JUSTICE GOSS: -- by looking at.

LETBY: Yes.

MR JOHNSON: By looking at the face and the hands.

MR JUSTICE GOSS: Right.

MR JOHNSON: Is that right?

LETBY: It's monitoring the whole situation of the baby, as I said, how they handle, how they look, how they feed.

MR JUSTICE GOSS: But it's visual, it's not electronic, it's not through a device?

LETBY: No.

MR JUSTICE GOSS: Right.

MR JOHNSON: Well, there was an electronic monitor on [Baby I], wasn't there?

LETBY: No, there was an apnoea alarm.

JOHNSON: Is that not a type of monitor?

LETBY: Yes.

MR MYERS: I think in fairness to Ms Letby, it's an awful lot for everyone to remember. I've been able, in the 2 minutes we've been going through this, on this occasion, to look back at the evidence of Dr Newby who was asked about this and it's in the transcript on 1 February, page 51. She was asked:

"Question: If a baby's on antibiotics, what is the practice, so far as you understand? The baby should continue to be monitored so far as heart rate and respirations are concerned?

"Answer: I would expect that to be the case, yes."

So she explained there should be monitoring of those items. She goes through -- she's asked as a general matter:

"Question: You'd expect that heart rate and respirations would be recorded whilst a baby's on antibiotics?"

"Answer: Yes, because being on antibiotics would imply you were worried about that baby."

And we then went through the charts to see the course of antibiotics that [Baby I] was on at that time. So I apologise for getting up as I do at this point, but having gone to that evidence, that's what this relates to. It may be quite difficult to think back in the moment for anyone, including Ms Letby, to the specifics of how that came out in the evidence and that's why I'm pointing that out.

MR JUSTICE GOSS: Yes. Thank you, Mr Myers. Don't apologise for it. I wasn't questioning anything. I was just trying to identify the terms that are being used here. I understand the point about if a baby's on antibiotics they should be monitored, in other words their readings should be looked at from the monitors for up to 48 hours after the course of antibiotics ends.

What I'm concerned about is when Ms Letby is using the word monitoring or "should have been monitored" or something, I want to know exactly what her definition of monitoring is in the circumstances.

MR MYERS: I understand, my Lord.

MR JUSTICE GOSS: That's all I'm trying to clarify because I'm confused. I may be alone in this confusion but others may be confused. I just want to know what it was. So I was just inviting clarification.

MR JOHNSON: It's very helpful. I couldn't understand what was being said.

MR MYERS: I hasten to add, I looked for that, not as a result of what your Lordship was asking, but as a result of what the prosecution was asking and it can take a while to go back through the massive amount of evidence to find it and that's what it relates to.

MR JUSTICE GOSS: Exactly, I understood that. We've just had the word "monitoring" quite a lot coming up recently in relation to this and I was getting a bit confused about precisely what's meant.

Ms Letby, do you want to explain -- do you understand what I'm trying to identify, the difference between someone actually monitoring the readings on a monitor, looking at the readings

and making notes of all the readings, and monitoring, as in just keeping an eye on a baby, just to look and see what they're looking like and that sort of thing?

LETBY: Not just how they look but how they handle, how they feed, how they wake --

MR JUSTICE GOSS: Can you explain -- when you use the word "monitoring" which you are meaning in the particular circumstances?

LETBY: Monitoring would include observations on a monitor if they were on a monitor. If they're not then it would be visual. And as I've said, looking at behaviour, how they feed --

MR JUSTICE GOSS: All right, thank you. Now that we understand -- well, I understand what you're meaning in relation to this, I'll let Mr Johnson continue with questions and I apologise for having interrupted but I was just becoming increasingly confused.

MR JOHNSON: As was I.

The easiest way to cut to the quick on this may be to look at the charts in divider 12, please. If you go to the second section behind divider 12, please, and it should begin with page J14719.

Here we see the observation chart, don't we?

LETBY: For 9 October, mine is.

JOHNSON: It begins on the 9th but it then runs to the 10th.

LETBY: Yes.

JOHNSON: Then to the 11th, then to the 12th, and then to the 13th, actually, at 01.30 in that far right-hand column.

LETBY: Yes.

JOHNSON: We see that monitoring for [Baby I] terminated -- or the last time before this incident that there had been any monitoring was at 21.25 on 10 October.

LETBY: Yes.

JOHNSON: Therefore, there had been a very clear 48 hours between the end of monitoring of [Baby I] and this incident, so even if monitoring was ended at the point that antibiotics were ended, there had been a good 48 hours between that and this event.

LETBY: If this is correct, then yes.

JOHNSON: Yes. Well, have you any reason to doubt the information in this document?

LETBY: No.

JOHNSON: No. If you turn over, the next few entries have been completed by you, haven't they?

LETBY: Yes.

JOHNSON: Well, you mentioned feeding. If we move on in the document in this section of the documents to 14791, at this point does it appear that [Baby I] was being fed by tube or by bottle?

LETBY: Sorry, I'm just locating that page.

JOHNSON: 14791. It's about three pages on from where we just were.

LETBY: Yes, thank you.

JOHNSON: Actually, 14789 may be better, just the page before in this document. Sorry, my mistake. So [Baby I] had been receiving bottles; is that right?

LETBY: Sorry, can you just tell me which page?

JOHNSON: Sure, 14789. It's on the screen if you look. Okay?

LETBY: Yes.

JOHNSON: So the next due feed is just over the page at 14790.

LETBY: Yes.

JOHNSON: And that was given via bottle, wasn't it?

LETBY: Yes.

JOHNSON: By Ashleigh Hudson?

LETBY: It's not signed, but I'm assuming Ashleigh, yes.

JOHNSON: She was certainly the designated nurse.

LETBY: Yes.

JOHNSON: So now we've established what the position actually was with feeds and with monitoring, why was Ashleigh too inexperienced to be looking after [Baby I]?

LETBY: I don't think Ashleigh had a lot of experience in recognising changes in babies potentially.

JOHNSON: You don't think she had experience in recognising changes in babies potentially?

LETBY: Yes.

JOHNSON: What does -- it may be me again but what does that mean?

LETBY: I think the more experience you have, the more attuned you are to noticing any behaviour changes, colour changes, anything with the baby.

JOHNSON: All right. Well, let's just go back to your defence statement, please, to paragraph 117. There you say:

"I think that Ashleigh and I both went to nursery 2 at the same time, although I cannot remember why."

LETBY: Yes.

JOHNSON: Having reviewed all the documents in the case, have you found a reason why you would have needed to have been in nursery 2?

LETBY: No.

JOHNSON: I will come back to this:

"The room was not in complete darkness, no nursery would be. The lights would have been dim and we turned the lights up. I think we both walked into the nursery together."

So were you saying there that you put the lights on before you went into the nursery?

LETBY: Yes, my memory is that the lights were on a dimmer switch and we turned those lights up when we entered.

JOHNSON: So before you saw [Baby I]?

LETBY: That was -- yes, at the time, yes.

JOHNSON: That's what you're saying in the defence statement, isn't it?

LETBY: Yes.

JOHNSON: We'll come to the differences and how they can be explained in due course:

"I don't think I was at the door talking with Ashleigh."

LETBY: No, that's right.

JOHNSON: Well, is that your case now or was it then or have you changed your mind?

LETBY: No, I believe Ashleigh's evidence was I was stood in the doorway when she arrived and I do not think that is the case. I think we both went together.

JOHNSON: "I thought [Baby I] looked pale. When I went over to the cot I could see she was white and gasping for air."

Is that accurate?

LETBY: Yes.

JOHNSON: "We pulled everything off her, took the Neopuff and called for the doctor to come from nursery 1."

LETBY: Yes.

JOHNSON: "I assisted with the resuscitation."

LETBY: Yes.

JOHNSON: Then finally:

"The apnoea alarm hadn't gone off but the alarm would only go off if there had been no movement for 20 seconds and [Baby I] was gasping and therefore moving."

LETBY: Yes.

JOHNSON: So that's a summary, first, of your evidence, then of the defence statement, and I'll ask you the questions I always ask you now. Are you saying that staffing levels caused or contributed to the life-threatening desaturation suffered by [Baby I] on 13 October?

LETBY: No, I'm not, no.

JOHNSON: Are you saying that medical doctors' incompetence contributed to her desaturation?

LETBY: No.

JOHNSON: Are you saying nursing incompetence contributed?

LETBY: No.

JOHNSON: Are you saying anybody made a mistake in the lead-up to [Baby I]'s desaturation on 13 October?

LETBY: No.

JOHNSON: Can we go to the population distribution, please, at tile 31 of [Baby I] 2. We can see, can't we, that you had a child AM in nursery 1?

LETBY: Yes.

JOHNSON: Laura Eagles had the other two children?

LETBY: Yes.

JOHNSON: Ashleigh Hudson had all three in nursery 2, who included [Baby I] and [Baby G]?

LETBY: That's right.

JOHNSON: Your friend Janet Cox had two in nursery 3?

LETBY: Yes.

JOHNSON: And one in nursery 4?

LETBY: Yes.

JOHNSON: Any problem with staffing that night?

LETBY: I can't comment on what the ratio of staff was for the other babies, but I can't say what all of their acuity was at that time to know.

JOHNSON: Well, part of the documents you've received for this trial is the material underlying the neonatal review, isn't it?

LETBY: What do you mean, sorry?

JOHNSON: The statements and -- sorry, not the statements, the charts and the nursing records that are relevant to the material that is contained within the neonatal review.

Let me give you -- it may be easier, rather than you trying to work out what I'm saying, if we just look at the neonatal review for [Baby I]. [Initials of Baby I] 2. It's in the big white file that --

LETBY: Oh sorry.

JOHNSON: It's all right. The big white file underneath jury bundle 2. If we go to [Initials of Baby I] 2, which is event 12, [Baby I].

LETBY: Yes.

JOHNSON: On page 2 we have the same population distribution that we've just been looking at in the sequence of events.

LETBY: Yes.

JOHNSON: Then if we go to page 3 and through to page 7, where there are records from which all this information has been obtained relating to all these children. You have had an opportunity to look at them, haven't you?

LETBY: Yes.

JOHNSON: Yes. And so that material gives your experienced eye the capacity to assess the acuity of a specific child, doesn't it --

LETBY: Yes.

JOHNSON: -- to a degree --

LETBY: Yes.

JOHNSON: -- because you can look at how often each child is being observed --

LETBY: Yes.

JOHNSON: -- what sort of observations are being taken, whether a child is being fed by bottle or by drip or bag or TPN or whatever it is and that sort of thing?

LETBY: Yes.

JOHNSON: So that all helps you to make that assessment, doesn't it?

LETBY: Yes.

JOHNSON: So with that in mind, therefore, you do have information which enables you to assess the acuity of specific children if you wish to?

LETBY: Yes.

JOHNSON: Yes. So with that in mind, are you saying that there is any issue on this shift with the care or the ratio of staff to [Baby I]?

LETBY: No.

JOHNSON: Thank you. Do you remember Ashleigh giving evidence on 27 January?

LETBY: Yes.

JOHNSON: Do you dispute what she said, that [Baby I] was prospering?

LETBY: As in she was doing well at that time?

JOHNSON: Yes, she was doing well.

LETBY: No, I agree with that, yes.

JOHNSON: You've seen the chart now. Do you dispute that monitoring had been scaled right back for [Baby I]?

LETBY: Yes -- I mean, yes, it had, yes.

JOHNSON: And as we have seen, the probe or the monitor, whether it's a Masimo or Phillips, had been removed from the beginning of the morning shift on 11 October?

LETBY: Yes.

JOHNSON: And that is a good sign, isn't it?

LETBY: Yes.

JOHNSON: The feeding chart that we just looked at in hard copy, which is page 14790, shows that at 01.30 hours, in other words a short time before she collapsed, an hour and 50 minutes before she collapsed, [Baby I] had a full bottle?

LETBY: Yes.

JOHNSON: She was breathing for herself in air?

LETBY: Yes.

JOHNSON: And, we know from other evidence, was demanding feeds?

LETBY: Yes.

JOHNSON: And that is all a good sign as well, isn't it?

LETBY: It is, yes.

JOHNSON: Do you accept what Ashleigh Hudson said about having left nursery 2 to help Laura Eagles in nursery 1?

LETBY: I have no memory of that, so I can't comment on whether Ashleigh did or didn't do that.

JOHNSON: Well, whether or not you want to comment on it, do you dispute it?

LETBY: I have no memory, so I can't say whether that happened or not. I was not a part of that.

JOHNSON: All right. Well, you see, she said that she asked somebody to look after or keep an eye out in nursery 2.

LETBY: Yes.

JOHNSON: Presumably, you will say you just don't remember?

LETBY: That she asked me to look after [Baby I]?

JOHNSON: Well, asked anybody to start with.

LETBY: I can't recall specifically, no.

JOHNSON: No. It would be extraordinary, wouldn't it, if given that she had three -- all three children in nursery 2, if she didn't ask somebody to keep an eye out?

LETBY: Yes.

JOHNSON: Yes. She was coming into nursery 1, wasn't she, to help Laura Eagles?

LETBY: That's her evidence, yes.

JOHNSON: Yes. Laura Eagles was also helping with the job that needed to be done?

LETBY: Yes, from Ashleigh's statement, yes.

JOHNSON: Well, from her evidence as well?

LETBY: Yes.

JOHNSON: Did you swap with Ashleigh as she came in and you went out?

LETBY: I don't recall assisting with a cannulation process, no.

JOHNSON: Well, you wouldn't need three nurses to assist with a cannulation, would you?

LETBY: Depending -- I can't say that you definitely wouldn't, but two should be enough, yes.

JOHNSON: So would it make sense for you to have been the nurse that left nursery 1 to fill in for Ashleigh?

LETBY: I can't comment. I myself had a baby in nursery 1.

There was also Caroline Oakley available on the unit.

I can't say which one of us was asked to look after nursery 2.

JOHNSON: It's just when you were being asked -- you say you can't remember now, but when you were being asked by your counsel about this, you said you were not the person who was asked to fill in for Ashleigh in nursery 2.

LETBY: No, I don't believe I was.

JOHNSON: How can you say that if you can't remember?

LETBY: Because I think I would know if somebody had directly handed over care to me of three other babies in a different nursery.

JOHNSON: And how would you know?

LETBY: I would remember that and I would have done more work with the babies and I would have had to have handed over my baby in nursery 1.

JOHNSON: All right. Each time you cover for each other, you have a formal handover, do you?

LETBY: No.

JOHNSON: Well, what would we expect to see in the records of the three children in nursery 2 that we fail to see? What would we expect to see if their care had been handed over to you?

LETBY: Only if anything was done at that point of needing to be handed over.

JOHNSON: We can see what you were doing, though, can't we, from the neonatal review? If we go to [Baby I] 2, page 3 of 7, please. Just go from line 44 down, please, Mr Murphy.

There we see at 46 and 47 the observations and feed that were carried out with [Baby I] by Ashleigh Hudson at 01.30, don't we?

LETBY: Yes.

JOHNSON: You can confirm from the paper in front of you that the last thing you're recorded as doing before that is at line 36 at 01.05?

LETBY: Yes.

JOHNSON: And the next thing after [Baby I] was fed is at 02.00 for AM when you took AM's observations?

LETBY: Yes.

JOHNSON: Those of us that have the paper can confirm that AM was your child in nursery 1?

LETBY: Yes.

JOHNSON: Who, we infer, was on hourly observations from what we see in lines 65 and 66; is that right?

LETBY: Yes.

JOHNSON: If we turn over to 67 and 68, we see that as well as the observations at 03.00 hours, there you are with Laura Eagles administering some medication to AM?

LETBY: Yes.

JOHNSON: So it follows, doesn't it, that so far as the children, the other children, in nursery 2 were concerned, there's no record of you doing anything in that time?

LETBY: No.

JOHNSON: And so far as other people are concerned, we've got JF and [Baby G]. We see that JF was being given medication by Dr Clegg at lines 48 and 49 and by Ashleigh Hudson and Caroline Oakley at 59 and 60.

LETBY: Yes.

JOHNSON: Together with Dr Neame's notes at 3 am and some observations taken by Ashleigh Hudson, also at 03.00 hours; is that right?

LETBY: Yes.

JOHNSON: It looks from all that information, doesn't it, like having sorted out the child -- well, having fed -- sorry, no. Having sorted out the child JF or sorted out the observations at line 62 at 3 am, 03.00 hours, Ashleigh Hudson then went to nursery 1?

LETBY: Yes.

JOHNSON: And you were doing nothing else at this time, were you, other than, at the same time, the medication with Laura Eagles for your child, AM?

LETBY: Yes.

JOHNSON: And that's why I'm suggesting it makes perfect sense for you to have swapped with Ashleigh.

LETBY: Well, I don't recall that happening.

JOHNSON: She said, Ashleigh Hudson, that when she returned with milk for one of her children, she was preparing it and you were standing in the doorway. Do you remember that?

LETBY: Yes.

JOHNSON: And it was at that point that you said, "Don't you think [Baby I] looks pale?"

LETBY: Yes.

JOHNSON: How long after arriving did you notice that [Baby I] was pale?

LETBY: I can't put a definitive time, but it was very quickly.

JOHNSON: No one's going to hold you to the second, but give us an estimate, please.

LETBY: From my memory, we had just arrived at the nursery when I looked across and noticed.

JOHNSON: Were you helping her with the milk?

LETBY: Not from my memory, no.

JOHNSON: You wouldn't need to, she wasn't that inexperienced, was she?

LETBY: No.

JOHNSON: So why -- if you weren't the person that had swapped with Ashleigh into nursery 2, what on earth were you doing going to nursery 2 with Ashleigh?

LETBY: I don't think there's anything sinister in that. At night we talk to each other, we go round the unit to different nurseries. That is not an unusual thing to happen.

JOHNSON: No, but the unusual thing is that, yet again, when a child collapses, you are the person that is there.

LETBY: I don't agree that I was looking after [Baby I] at that point.

JOHNSON: The lights were off, weren't they?

LETBY: I can't say.

JOHNSON: Has your memory improved since you were interviewed?

LETBY: No.

JOHNSON: Can we go to [Baby I]'s interviews, please. Sorry, it's number 1, please. I want to start with the very first interview, please, which was 4 July 2018. Actually, I say page 13 because that's the bit I want you to concentrate on, but let's put it in context by going to page 12, please.

Towards the bottom of the page you had told the police that following [Baby I]'s collapse, you had taken over her care.

LETBY: Yes.

JOHNSON: The police asked you why that was and you said:

"Because staff Nurse Hudson was a junior band 5 that couldn't care for intensive care patients, which [Baby I] had then become."

All right?

LETBY: Yes.

JOHNSON: Then they're referring, are the police, to notes, which we'll come to, but they say this:

"In your notes you've put 'noted to be pale'. Can you elaborate at all on that? Can you expand any observations at all?"

And what is your response, please?

LETBY: "Some of it, when we went into the nursery, put the light on -- the lights aren't on in the nursery at night and we put the lights on for something and I noticed -- I looked over at [Baby I] and I noticed that she was pale in colour in the cot."

JOHNSON: Right. This is not what you said to the jury, is it?

LETBY: I cannot definitively say what was happening with the lights, no. This is my memory at that time.

JOHNSON: No, but there's a more fundamental point here, isn't there? Time and time again, witnesses have been asked about whether lights are on or off and what you are saying to the police here, not because they are leading you into it, when they asked you about the lighting in nursery 2, you are telling them that the lights are off, aren't you?

LETBY: Yes. And that's how I believed it to be, that we went to the nursery and put the lights on.

JOHNSON: But why have you repeatedly told the jury that the lights are never off?

LETBY: So the lights are never off completely: the lights are on a dimmer switch and there's still enough light from the unit to illuminate the nursery.

JOHNSON: Why don't you say "turn up the dimmer switch" then instead of "put the light on"?

LETBY: I don't know. There is no switch to flick the light on.

It's on a dimmer so that's the only way you can put the light on.

JOHNSON: Page 29, please, which is the third interview on 11 June 2019. At the bottom there, there's a six-line statement by one of the police officers that you agree with.

LETBY: Yes.

JOHNSON: And the statement says:

"With reference to you..."

Well, going further up for context, they're asking you about 13 October. They repeat to you what you had said in your previous interview and say:

"With reference to you commenting that [Baby I] was pale, you said, 'When we went into the nursery, put the light on -- the lights aren't on in the nursery at night and we put the lights on for something and I noticed'..."

And you say yes. Why don't you tell them about the dimmer switch and the lights being down low there?

LETBY: I don't know.

JOHNSON: They repeat that over the page on page 30. And yet again you confirm?

LETBY: Yes.

JOHNSON: Are you trying to massage the evidence to make it a bit better for yourself by saying the lights were on low?

LETBY: No.

JOHNSON: So if the lights were off, where had you just come from into that nursery, into the doorway of that nursery?

LETBY: I don't know.

JOHNSON: Well, from the corridor perhaps?

LETBY: I can't say.

JOHNSON: Well, I'm sorry, but how do you get physically to the doorway of nursery 2?

LETBY: I could have come from the corridor straight ahead from nursery 1 or I could have come from the lower half of the nursery by 3 and 4 or from the nurses' station.

JOHNSON: Whether you come from the nursing station, from a different nursery or you're just in the corridor, you've always got to go via the corridor, haven't you?

LETBY: Yes.

JOHNSON: Are the lights on in the corridor?

LETBY: Yes.

JOHNSON: If you go from a light corridor into a dark nursery, does that make your eyesight really good?

LETBY: I don't understand.

JOHNSON: Oh, I think you do. What effect does coming from a bright corridor into a very dim nursery have on your eyesight?

LETBY: I don't know.

JOHNSON: You really don't know?

LETBY: No.

JOHNSON: You are a nurse. Everybody knows, don't they? If you go from bright into dark, what effect does that have on your capacity to see in the dark?

LETBY: It would depend on the brightness of the light, but, yes, you wouldn't be able to see as well.

JOHNSON: No. And yet you noticed it straightaway?

LETBY: Yes.

JOHNSON: And the reason for that is because you had caused what it was you were purporting to notice, isn't it?

LETBY: No.

MR JOHNSON: My Lord, if we're going to have a break...

MR JUSTICE GOSS: We are going to have a break so we'll have it now. Thank you very much. 15 minutes, please.

(3.20 pm)

(A short break)

(3.35 pm)

MR JOHNSON: I'm going to show you a picture that Ashleigh Hudson annotated. It's covered by agreed fact 51. The reference is J25368.

Do you agree that that is an accurate representation of the view?

LETBY: No.

JOHNSON: No? In what respect is it inaccurate?

LETBY: There would be more light visible.

JOHNSON: More like what, sorry?

LETBY: There would be more light -- the room would be lighter and the cot would potentially be nearer to the workbench.

JOHNSON: Would potentially be near to the work? Well, all sorts of things are potential. The cot potentially could be upside down. Where are you saying it was?

LETBY: I think it was nearer to the workbench than that.

JOHNSON: What is the purpose of that tent-like structure over the cot?

LETBY: To minimise bright light to the baby.

JOHNSON: How big are the hands of a child of [Baby I]'s age?

LETBY: Small.

JOHNSON: Tiny, aren't they?

LETBY: Yes.

JOHNSON: Her head is small as well, isn't it?

LETBY: Yes.

JOHNSON: And she was covered with a babygro?

LETBY: Yes.

JOHNSON: There was almost nothing to see, was there?

LETBY: No, just her hands and her face.

JOHNSON: Which would have been covered by that tent-like structure?

LETBY: Not entirely, no.

JOHNSON: Ashleigh Hudson was right, wasn't she, when she said you can't see anything from that doorway?

LETBY: No.

JOHNSON: Do you remember what you said to the police when they asked you about this?

LETBY: No.

JOHNSON: Let's go to the third of your interviews, it's [Initials of Baby I], page 34, please. This is the interview of 11 June 2019.

Look up from the bottom, it's about halfway down the page, 3100. This is the officer speaking to you:

"She [this is a reference to Ashleigh] says the first time she saw [Baby I] pale -- how could you see from the doorway that [Baby I] was pale without having the light on, Lucy? How did you know she was pale?"

And you said what?

LETBY: "Maybe I spotted something that Ashleigh wasn't able to spot. The rooms are never that dark that you would not be able to see the baby at all. There's always a level of light for that reason".

JOHNSON: You don't have better eyesight than Ashleigh, do you?

LETBY: No.

JOHNSON: At page 35, over the page, you were putting all this down to your greater experience; is that right?

LETBY: No.

JOHNSON: Well, the question is:

"How would you be able to spot the colouring then that Ashleigh couldn't if you were both stood at the same place?"

And your answer?

LETBY: That I had more experience so I knew what I was looking for or looking at.

JOHNSON: Ah, knew what you were looking for. What did you mean by that?

(Pause)

Your answer. You explain it.

LETBY: Yeah, I don't mean it like that. I'm finding it quite hard to concentrate on all of the dates at the minute.

MR JUSTICE GOSS: Right. Well, we would do about another half an hour this afternoon, but having observed the witness and given that it's been a long day for her in the witness box, I'll stop the proceedings this afternoon and we will resume on Tuesday morning at 10.30, please, members of the jury.

Please remember, as I'm sure you do, your responsibilities: keep your thoughts to yourselves unless you're all together and everyone can hear what someone else is saying about anything to do with this case. Obviously, you're free to speak in those circumstances but none other, and no communication with anyone about anything to do with this case by any means and no research.

Thank you very much indeed. 10.30 Tuesday morning.

(3.42 pm)

(The court adjourned until 10.30 am Tuesday, 30 May 2023)